

OPPORTUNITY REVIEW

DIRECT-TO- EMPLOYER GTM STRATEGY

Exploring a Trinity Health Michigan DTE Health Plan



An aerial photograph of the Indianapolis skyline at sunset. The Indiana State Capitol is prominent on the left, with its tall tower topped by the Spirit of Liberty statue. The city is filled with various buildings, including a large modern glass-walled building on the right. The sky is a mix of orange, yellow, and blue, indicating the time is either dawn or dusk. The foreground shows a park area with trees and a fountain.

KEY BENEFIT ADMINISTRATORS

- Founded in 1979, headquartered in Indianapolis
- Largest Independently-Owned Full-Service Third-Party Administrator
- Specialize in Administration of Self-Funded Plans and Population Health Management
- Leader in the Direct-to-Employer Movement
- We Know and Understand the Market
- Experience Working with All Professional Teams: Agents, Networks, MGAs
- The Key Family of Companies is a Vertically Integrated group of benefit related organizations
 - 3,000+ Corporate Clients
 - 3 Million Members under Management
 - 700 Employees

MARKET LEADER IN PRIVATE LABELED DTE PLANS





Trinity Health

DIRECT-TO-EMPLOYER (DTE)

Your Health Plan
Focused on a Direct
Relationship with
Self-Funded
Employers in *Your*
Market



YOU Pick the Name of the Health Plan



YOU Determine the Fees Charged



YOU Practice the Medicine



YOU Receive the Profits



WE Make it Successful

Key Benefit Administrators



Direct-to-Employer Strategy: POWERFUL EMPLOYER VALUE

- Reduced medical spend through lower unit costs and better integrated care
- “At the table” with Trinity Health MI
- Aligned financial incentives
- Complete transparency for unlimited visibility into health benefit plan expenses and cost drivers
- Aggressive stop-loss premiums and policy terms
- Increased employee satisfaction
- Improved cash flow & stabilization of future medical cost trend

WHY KBA?



OUR 'WHY': THE PHOTO MOMENT

THE KBA REWARD THROUGH SEEING A DIFFERENCE



Better Care, Lower Costs, Real Partnerships

- All Stakeholders at the Table (Employer, Provider, KBA, Broker/Consultant)
- Reviewing Real Progress (Improved Population Health and Lower Spend)
- Deep Loyalty Between Employer and Provider Organization Built on Value

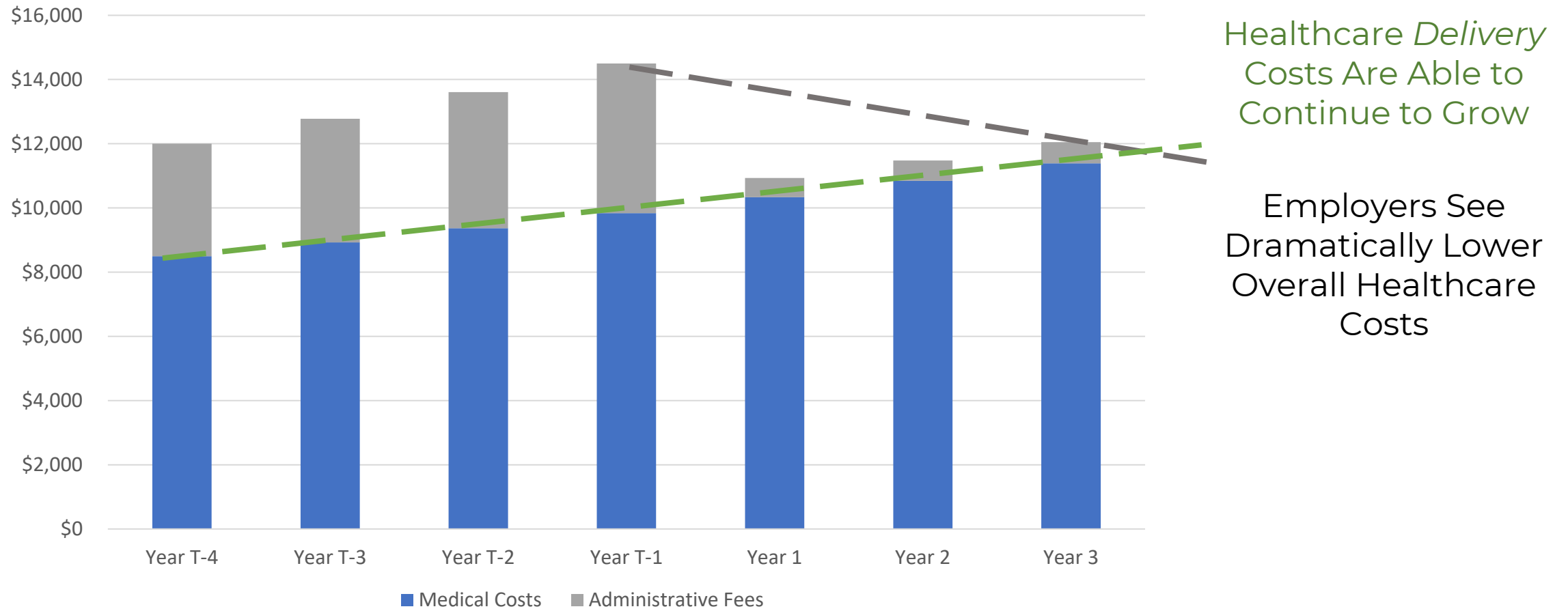


Our Founding and Heritage: 46 Years of Striving for Honesty in Healthcare

- **Founded Key Benefit Administrators in 1979:** After co-founding a life insurance agency in 1973 and Key Life Insurance in 1976, Larry Dust launched KBA in 1979 as a third-party benefits administrator
- **Sold to Blue Cross/Blue Shield of Indiana in 1985:** In 1985, he sold KBA to BCBS of Indiana (then part of the Associated Group)
- **Reacquired KBA in 1992:** Larry bought the business back from BCBS in 1992, returning it to independence
- **Built it into the largest independent TPA in the U.S.:** Under his leadership since the buyback, KBA has become the country's largest *independently owned* third-party administrator (and Population Health Administrator), managing millions of members
- **Never Forgotten Who Gives Healthcare:** KBA has never participated in reference-based pricing, does not take refunds, and delivers healthcare at 11% below market rates.

Key Benefit Administrators

THE WIN/WIN OF CUTTING OUT THE MIDDLEMAN

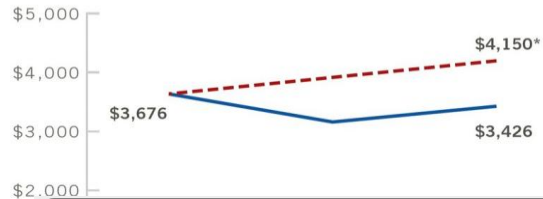


The **RED LINE** =
The year over year healthcare
component of the consumer price index

The **BLUE LINE** = What happens when you
manage your health on Main Street

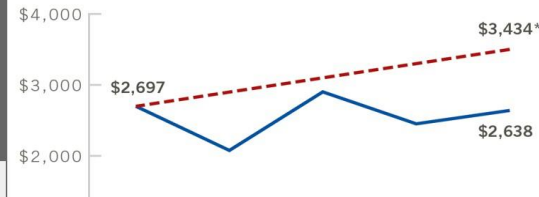
1 | THREE YEAR MEDICAL AND RX TREND ANALYSIS PMPY CLAIMS ONLY

Company 1 is a global engineering consulting firm headquartered in Indiana with 687 members. Since joining the program, Company 1 has been on a consistently favorable financial path and is currently **21% below trend**.



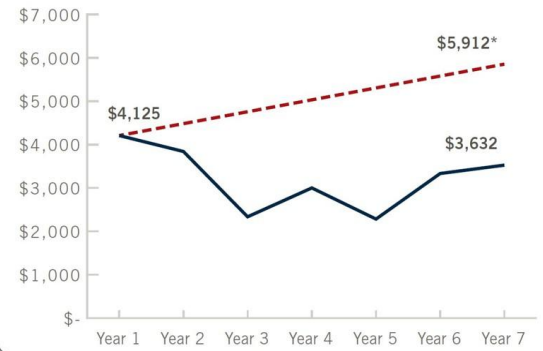
2 | FIVE YEAR MEDICAL AND RX TREND ANALYSIS PMPY CLAIMS ONLY

Company 2 specializes in transport refrigeration with 198 members. Company 2 has been on a consistently favorable financial path and are currently **30% below trend**.



3 | SEVEN YEAR MEDICAL AND RX TREND ANALYSIS PMPY CLAIMS ONLY

Company 3 is a nonprofit organization with 784 members. Since joining the program, Company 3 is **39% below trend**.



* U.S. Government Statistics

— ACTUAL — PROJECTED



American Health Data Institute

For our clients, 98% of our
Blue Lines are **BELOW** their
Red Lines!



American Health Data Institute

Financial Savings & Return On Investment

with the Health Care Nurse Navigator Chronic Disease Management Program

AHDI has five different statistically validated ROI studies performed in 2006, 2008, 2010, 2014 and 2021. All studies support a consistent and increasing reduction of medical spending between 11-17%.

2021 CDM Analysis Once Again Validates the Value of AHDI's CDM Program

Summary:
This study used claims on 100,000 plus CDM members with continuous enrollment over a two-year period.

Discussion:
A fixed effects least squares regression model was used in this analysis. The use of a fixed effects model controls for price variation between healthcare markets.

Findings:
Participation in the AHDI Chronic Disease Management program was associated with a continued average reduction in claims cost of 11% to 17%, at the 90% confidence interval. This results in an average Return on Investment of 14 to 1. These results are consistent with those of the prior five studies conducted on the CDM program.



36.5% OF AMERICANS DELAYING MEDICAL CARE DUE TO COST

The high cost of healthcare in the United States is a significant barrier to accessing necessary medical services. According to recent data, over a third of Americans have had to skip or delay their medical care in the past year due to the financial burden. This is a concerning trend that highlights the need for more affordable and accessible healthcare options.

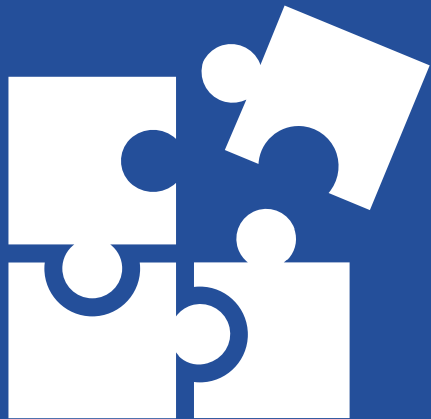
INTEGRATION WITH EPIC EHR SYSTEM

**KBA shares
evidence-based
standards of care for
27 chronic conditions
directly into Epic**



Hyperthyroidism: Yearly Clinical Evaluation, TSH and T4

THE MISSING PIECE TO VALUE-BASED CARE



- Patient engagement is the weak link in delivering value-based care; Trinity Health's clinical leadership and employer participation can drive better outcomes.
- KBA's proactive care-management team supplements your care-management efforts in outreach to clients.
- Trinity Health excels in Standards of Care completion rates for the Chronically Ill, improving care and costs.
- Integrating Trinity Health's managed care with KBA's programs positions Trinity Health as the value leader.

TRANSPARENT PBM INTEGRATION

- Support of Trinity Health's retail pharmacy(s)
- Optimize Trinity Health's 340B drug pricing program
- Allows Trinity Health to create its own PBM program if desired
- Custom telephone number with personalized scripting and separate call tracking & reporting
- Fully customizable member welcome booklet & materials
- White labeled formulary document and marketing materials

CUSTOM CLIENT SUPPORT:

- Specialized call center capabilities
- Fully customizable plan designs
- Operational access to the platform
- Customized process and functions to promote DTE strategy

CLINICAL FLEXIBILITY:

- Full portfolio of custom formulary offerings – with optimized formulary shield
- Clinical programs that drive towards overall DTE strategy
- Options for additional drug savings by driving members to lower cost alternatives
- Flexibility to customize or layer on utilization management programs

STOP-LOSS INSURANCE

THE KBA EXPERIENCE

- Deep relationships with 20+ stop loss underwriters and carriers
- Over \$150M in annual stop loss premium placement
- Comprehensive pricing and medical service analysis performed for each DTE health plan to ensure proper pricing
- KBA provides the administration required of the stop-loss markets:
 - Bordereau Reporting
 - Claim Reporting and Reconciliation
 - Evaluation of Networks
 - Care Management
 - Initial Underwriting
 - Negotiation of rates and terms
 - Plan Design Actuarial

Tokio Marine

Nationwide

Berkshire

Hathaway

Sun Life

Liberty Mutual

Granular (Google)

Companion Life

Crum & Forster

American Fidelity

RGA

DTE PROGRAM MARKETING & SALES

KBA WORKS WITH YOU TO MANAGE & SELL:

- Branding & Messaging
- Direct & Brokerage Sales Distribution – Focused Sales Representative
- Brochures, Web Site, Media, etc.
- Stop-Loss/Level Funded Down to 20 EE Lives
- Sell Through Established Brokers in Your Service Area

COMMUNITY BASED SALES STRATEGY



Locally based sales rep with strong community ties



Collaboration with DTE provider partners to identify strategic employers for early adoption and sales



Work with select broker distribution partners with aligned incentives and goals



Pipeline reporting and collaboration with our DTE provider partners



Marketing and promotion of the new option in the community

COMPLETE MARKETING SYSTEM

- Customized Branding and Messaging
- Customized Brochures
- Customized Web Site
- Targeted Employer Outreach
- Sales System (Social Media, Email, Text, etc.)



THE KBA HEALTHCARE CONVENIENCE CARD

Creating EPO Steerage in a PPO Network

The Healthcare Convenience Card is eligible to be used only for specific merchant codes.

1. To utilize at Specific Healthcare Providers
 - a. Designated Hospitals
 - b. Urgent Care Facilities
 - c. Physicians
 - d. Eligible Ancillary Providers
2. Out of Pocket Expenses Qualify
3. 0% Interest Rate
4. Employee Determines Repayment Schedules
5. To Qualify: Answer One Simple Question



KEY INCENTIVE PLAN (KIP):

BASE MEDICAL + BONUS BENEFITS = YOUR DEDUCTIBLE CAN SIMPLY VANISH

BENEFIT EXAMPLE

BASE MEDICAL BENEFITS		BONUS BENEFITS	
Deductible		Benefits Automatically Included In Your Plan, At NO Additional Cost To You!	
Single	\$2,500	Preferred Hospital Stay	5 Days at \$500 / Day
Family	\$3,500	Non-Preferred Hospital Stay	1 Day at \$50 / Day
Coinsurance	80% / 20%	OP Surgery	\$700 / 1 Day
Maximum Out-of-Pocket		OP Anesthesia	\$250 / 1 Day
Single	\$3,500	OP Radiology/X-Ray	\$200 / 1 Day
Family	\$7,000	OP Diagnostic Labs	\$100 / 1 Day
Chronic Disease Management Regimens of Care, Covered at 100%		Health Screening Benefit	\$125 / 1 Day
Healthcare Specific, Interest Free Convenience Card to Cover Out-of-Pocket Costs		Wellness Benefit	\$125/1 Day
		Critical Illness Benefit*	\$2,000 Lump Sum
		*Critical Illness Benefits pays for 1st Diagnosis: Heart Attack, Stroke, Internal Cancer	

CLAIM EXAMPLE

“How Your Deductible Can Vanish”	
CLAIM EXAMPLE: Heart Attack, with Surgery	
OP Radiology/X-Ray	\$200
OP Diagnostic Labs	\$100
Preferred Hospital Stay, 5 Days at \$500/Day	\$2,500
Critical Illness (Heart Attack Benefit)	\$2,000
Health Screening Benefit (Stress-Test)	\$125
Wellness Benefit (Telephonic Coaching)	\$125
TOTAL BONUS BENEFIT PAYABLE	\$5,050
Less Your Maximum Out-of-Pocket Cost	(\$3,500)
Additional Benefit, Payable Directly to You!	\$1,550!

THE BENEFIT STEERING THE PATIENT TO TRINITY HEALTH

What Will this Claim Cost Me?



Trinity Health MI patients will often receive **cash, not bills** after care episodes!

KEY BENEFIT
ADMINISTRATORS

Explanation of Benefits	
Heart Attack, Tests Run, Hospital Stay	
THE CHARGES:	
Medical Bills	\$25,000
Healthcare Credit Card Balance (the Out-of-Pocket You Paid)	\$3,500
Once your Out-of-Pocket is paid, the rest of your eligible charges that year would be paid at 100%	
THE CREDITS:	
Out Patient Testing	\$300
Health Screening Benefit	\$125
Wellness Benefit	\$125
Hospital Admission Benefit	\$2,500
Critical Illness Benefit	\$2,000
Benefit Total:	\$5,050
Healthcare Credit Card is Paid Off	(\$3,500)
Balance Paid to You:	\$1,550

NETWORK MANAGEMENT AND OPERATIONS



The Healthcare Provider Landscape Challenge

The Michigan healthcare market presents a complex web of care providers, specialty groups, and healthcare facilities that can overwhelm patients, employers, and payers alike.

Market Complexity

- Hundreds of independent provider provider groups
- Multiple health system affiliations
- Varying quality and pricing structures structures
- Limited transparency in outcomes

The Challenge

Without strategic curation and management, this overwhelming number of number of care groups creates confusion, confusion, inefficiencies, and suboptimal suboptimal outcomes for all stakeholders. stakeholders.

The solution: A carefully managed, high-value network.



Our Goal in Partnering With Trinity Health



Build the Highest-Value Network

Create the Michigan area's premier healthcare network that delivers exceptional quality care with market-leading pricing, setting the gold standard for value-based healthcare delivery.



Enable Fastest-Possible Growth

Leverage our proven methodologies and established Trinity Health Alliance relationships to accelerate network expansion, ensuring rapid market penetration and comprehensive coverage across Michigan.



Vault Trinity to Competitive Advantage

Position Trinity as the anchor and leader of this transformative network, creating sustainable competitive advantages that drive market leadership and long-term success.

"Value is high-quality care at a great price – this principle guides every decision we make in network development and provider selection."



HEALTHCARE
PARTNERSHIP

Networks Must Be Carefully Tended

Proven Network Leadership

Our leadership team brings unparalleled depth of experience to healthcare network management management and strategic development.

Each team member has over 25 years of hands-on experience in healthcare network operations, operations, provider relations, strategic planning and contracting.

Our Management Approach

- **Yearly contracting cycles** – ensuring competitive rates and terms
- **Network integrity monitoring** – maintaining quality and access standards
- **Provider relations management** – fostering strong, productive partnerships
- **Performance analytics** – continuous improvement through data-driven insights insights

We manage all the tending always in cooperation with the sponsoring provider organization.



Vehicle for Approving Network Changes: Touch Point Meetings

Regular touchpoint meetings serve as the strategic command center for network development, ensuring every addition aligns with organizational goals and market needs.

01

Network Analysis

Comprehensive evaluation of current network gaps, utilization patterns, and performance metrics to identify strategic opportunities.

02

Stakeholder Feedback

Regular collection and analysis of feedback from employers, brokers, and members to ensure network meets real-world needs and expectations.

03

Option Review

Systematic evaluation of potential provider additions, considering clinical quality, geographic geographic coverage, and financial impact.

04

Strategic Recommendations

Data-driven recommendations presented to leadership for informed decision-making on network expansion and optimization.

05

Contracting Execution

Upon agreement, we approach and negotiate with target provider groups to secure favorable terms and seamless integration.



Who Needs to Be in the Touchpoints

i Critical Success Factor: Dedicated system leaders who can make real-time decisions about network inclusion and exclusion criteria.

Decision-Making Authority

Touchpoint meetings require participants to advise network changes, contract terms (where terms (where possible), and strategic direction.

Meeting Structure & Frequency

- **Weekly meetings** for the first several months during initial network build-out
- **Monthly meetings** once network reaches maturity
- **As-needed sessions** for urgent opportunities or issues

Required Participants

- Network leadership with contracting authority
- Clinical quality decision-makers
- Financial/pricing stakeholders
- Strategic planning representatives

The group collectively decides the meeting structure and governance approach that works best for their organization.

Contract Negotiations & Risk Management



Comprehensive Negotiation Management

We handle all contract language, terms, and subsequent subsequent reimbursement negotiations, ensuring optimal optimal terms while protecting your organization from legal from legal and financial risks.



Price-Fixing Protection

By managing all rate negotiations directly, we create a protective barrier that keeps Trinity Health safe from potential price-fixing concerns and antitrust issues.



Annual Renewal Excellence

All negotiations are renewed yearly, and our strong track record of delivering excellent reimbursements to providers enables smoother renewals and stronger relationships.

Our Negotiation Advantages

Our extensive experience across multiple markets gives us unique insights into fair market rates and industry best practices. We leverage practices. We leverage data analytics and market intelligence to secure favorable terms for both the network and participating providers. participating providers.



Claims Processing Excellence

- Delivering a positive member and provider experience starts with accurate, efficient, and transparent claims processing
- **Key Benefit Administrators leverages:**
 - Proven technology
 - Streamlined workflows
 - A team dedicated to first pass accuracy
- **We Consistently:**
 - Exceed industry standard turnaround times and quality metrics
 - Reduce administrative burdens
 - Maintain high member and provider satisfaction
- **Strategic Partnerships:**
 - We partner with **Smart Data Solutions** for clearinghouse services to empower providers with electronic claims submission and transparency tools.
 - Currently 82% of our claim volume is received electronically.
 - Ongoing efforts to increase electronic submissions:
 - Analyze paper and non-electronic claims
 - Proactively engage providers for electronic claim setup
 - We also partner with top tier vendors to provide cost containment solutions to include:
 - Out of network discounts
 - High dollar bill reviews
 - Specialized networks for high-cost conditions such as transplants and dialysis.



Claims Statistics

Claims	Industry Standard	Corporate Goal	Self-Funded Medical Achieved*
Turn Around Time	10 calendar days	7 calendar days	4.25 calendar days
% Paid in 10 days	80%	85%	86.7%
Financial Quality	99%	99%	99.6%
Payment Accuracy	NA	95%	97.4%

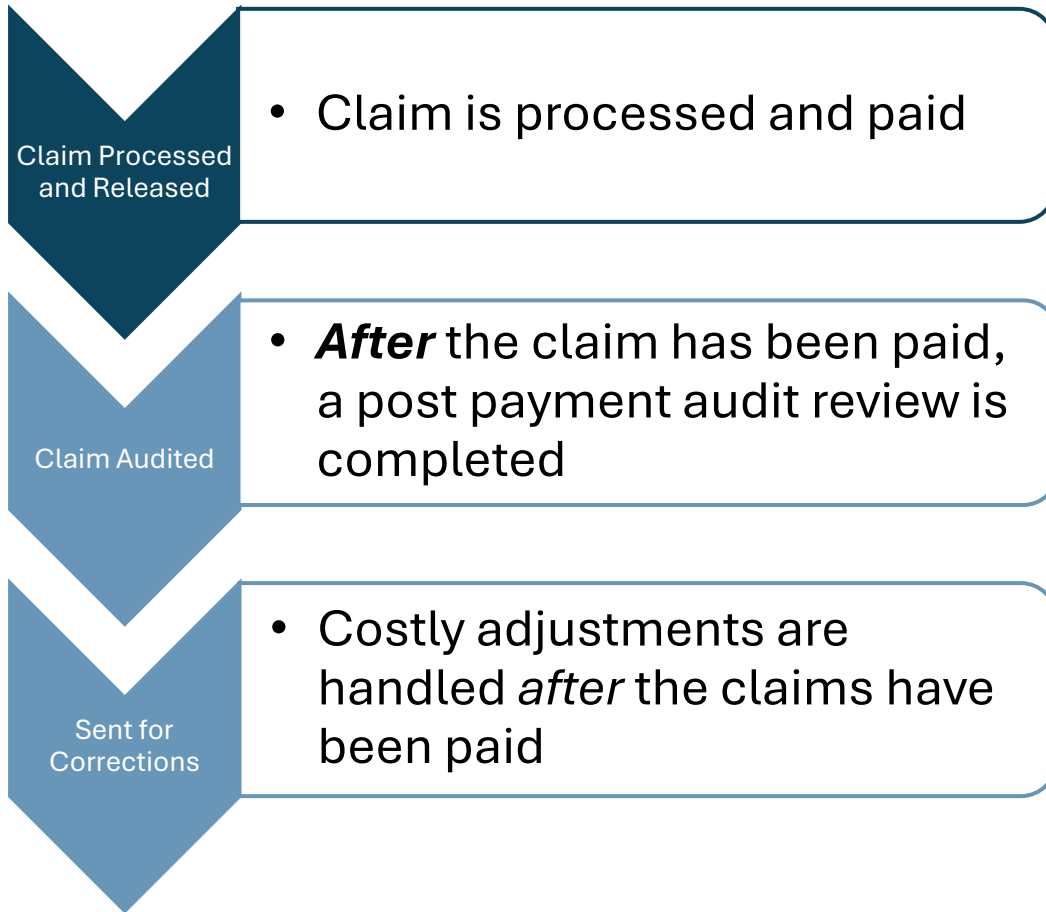
* Data achieved is for Jan-Jun 2025 calendar year

- Groups are provided with a dedicated specialist to handle escalated issues and concerns promptly.
- We offer comprehensive, interactive training for claims and customer service staff to prepare them for their roles, using a collaborative LMS system to ensure consistency and accuracy in KBA processes.



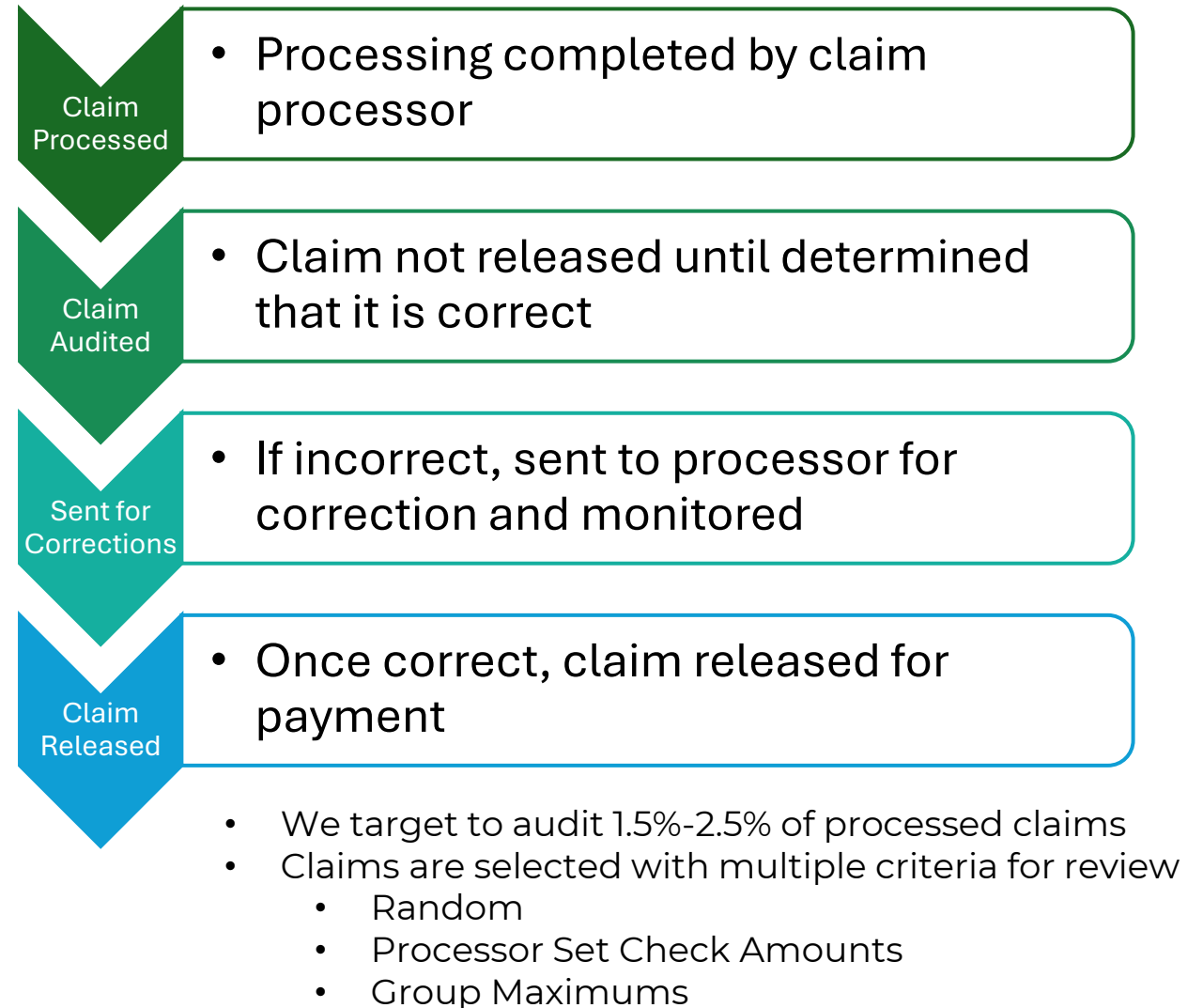
Industry Claim Audits

Post Payment Reviews



KBA Claim Audits

Pre Payment Reviews



KBA 2023-2024 Total Quality Management Statistics

300+
TOTAL QUALITY MEASURES

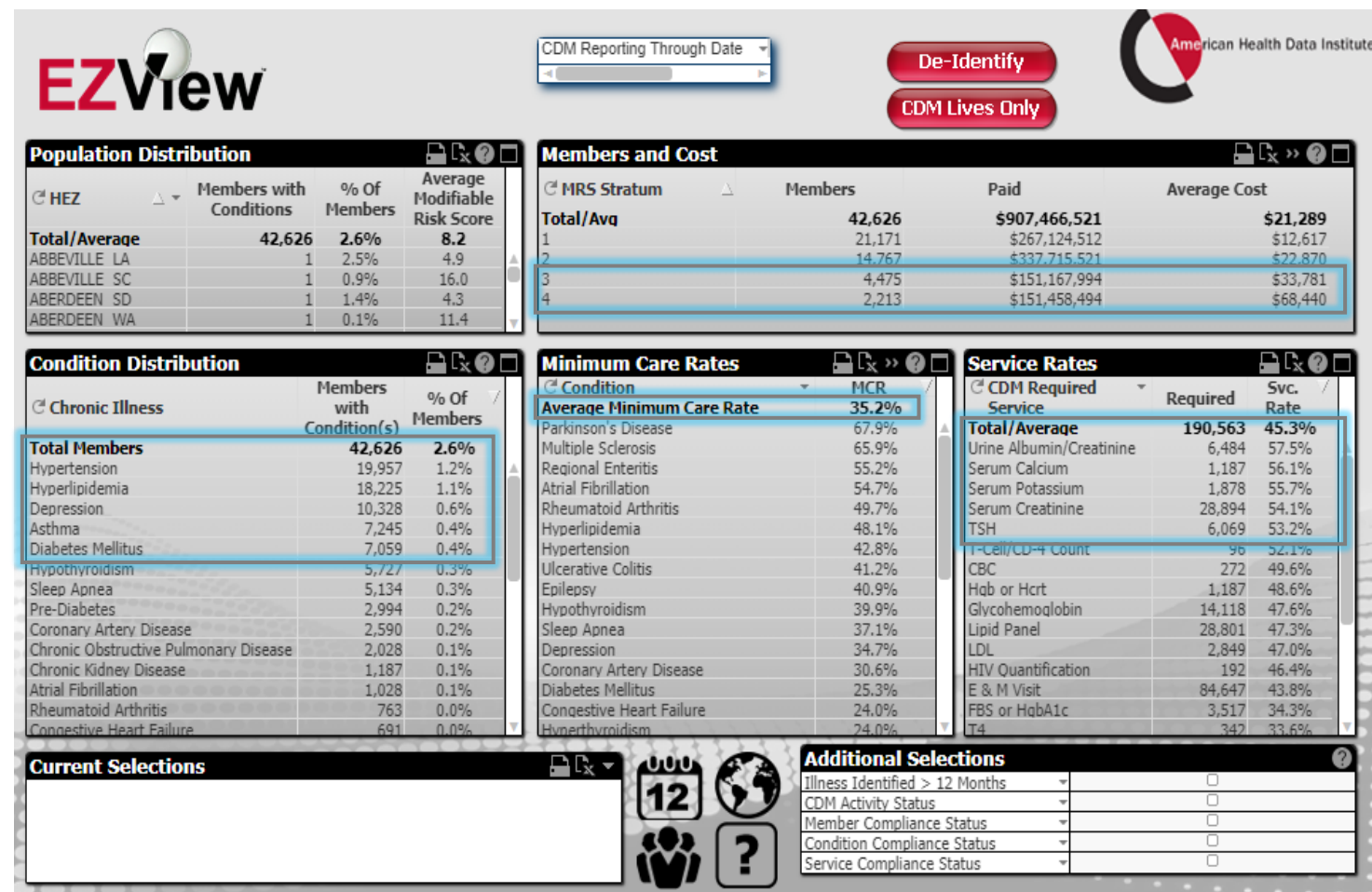
50,000+
UNIQUE AUDITS ANNUALLY

\$200M+
PREMIUM & CLAIMS REVIEWED ANNUALLY

New Business / Renewals					Enrollment Center				
New Case Accuracy	2023 KBA 99.86%	2024 KBA 100.00%	INDUSTRY N/A	GOAL 99.75%	Soft Skills Audit	2023 KBA 99.43%	2024 KBA 99.22%	INDUSTRY N/A	GOAL 98.00%
Renewal Case Accuracy	2023 KBA 99.96%	2024 KBA 100.00%	INDUSTRY N/A	GOAL 99.75%	Procedural Skills Audit	2023 KBA 98.26%	2024 KBA 98.23%	INDUSTRY N/A	GOAL 97.00%
Fulfillment Materials Audit	2023 KBA 99.99%	2024 KBA 99.98%	INDUSTRY N/A	GOAL 100%					
Billing & Eligibility Quality					Claims Quality				
Eligibility Accuracy	2023 KBA 100.00%	2024 KBA 100.00%	INDUSTRY N/A	GOAL 99.75%	Turn Around Time (Days)	2023 KBA 3.79	2024 KBA 6	INDUSTRY 10	GOAL 7
Plans/Rates/Coverage Audits	2023 KBA 100.00%	2024 KBA 100.00%	INDUSTRY N/A	GOAL 99.75%	% Paid within 10 Bus Days	2023 KBA 88.90%	2024 KBA 87.34%	INDUSTRY 80.00%	GOAL 85.00%
Cash Receipts Logging	2023 KBA 99.97%	2024 KBA 99.78%	INDUSTRY N/A	GOAL 98.00%	Financial Accuracy	2023 KBA 99.30%	2024 KBA 99.44%	INDUSTRY 99.00%	GOAL 99.00%
Cash Posting Accuracy	2023 KBA 99.96%	2024 KBA 99.78%	INDUSTRY N/A	GOAL 97.00%	Procedural Accuracy	2023 KBA 99.00%	2024 KBA 99.16%	INDUSTRY N/A	GOAL 95.00%
					% of Claims Audited	2023 KBA 3.77%	2024 KBA 3.80%	INDUSTRY 1%	GOAL 3%
Customer Experience Quality									
Average Speed to Answer	2023 KBA 22 sec	2024 KBA 44 sec	INDUSTRY <45s	GOAL <30s	% Answered w/in 45 Sec	2023 KBA 84.90%	2024 KBA 75.00%	INDUSTRY 80-60%	GOAL 80-30%
Soft Skills Audit	2023 KBA 99.50%	2024 KBA 99.54%	INDUSTRY N/A	GOAL 98.00%	Procedural Audit	2023 KBA 99.22%	2024 KBA 99.10%	INDUSTRY N/A	GOAL 97.00%
Email Correspondence Audit	2023 KBA 100.00%	2024 KBA 100.00%	INDUSTRY N/A	GOAL 98.00%	Web Chat Corresp. Audit	2023 KBA 99.64%	2024 KBA 99.50%	INDUSTRY N/A	GOAL 98.00%
Abandonment Rate	2023 KBA 1.00%	2024 KBA 1.78%	INDUSTRY <5%	GOAL <1%					

TRANSPARENT ANALYTICS ACCESS

- EZView analytics platform provides instant access to critical analytics insights.
- Filter to a population by data source, group, plan, or geographic location.
- Identifications of chronic conditions, patient acuity, compliance with nurse coaching, regimens of care, steerage rates or claims costs.



DTE Implementation Team

The DTE Implementation Team is responsible for the overall management of the DTE project plan for the DTE Network and KBA Administrative Operations.

We are always excited about new partnerships and look forward to developing a solid foundation to build upon.



Christine Vinesett

Director, Implementation and Network

I joined Key Benefit Administrators in 2000 and have been in the Health Insurance Industry for 20+ years. My focus has been Leadership in Operations, Carrier and Provider Network Relations and Implementation Management.

Office: (803) 323-7974 | Email: Christine.Vinesett@keybenefit.com



Julie Caskey

Network Operations Lead

Julie Caskey joined Key Benefit Administrators (KBA) in 2021. Julie brings an extensive background with 30+ years in the Health Insurance Industry. Julie's focus has been in Provider Network and Relationship Management.

Office: 317-712-4003 | Email: Julie.Caskey@keybenefit.com



Dejanetta (Deja)
Pettaway

DTE Implementation and Relationship Coordinator

Deja joined Key Benefit Administrators in 2023 and has been in the Health Insurance Industry for 10 years. Deja's focus has been in Provider Network and Relationship Management.

Office: 803-396-4836 | Email: Dejanetta.Pettaway@keybenefit.com



Darlene Grindstaff

DTE Implementation and Relationship Coordinator

Darlene Grindstaff joined Key Benefit Administrators in 2014 and has been in the Health Insurance Industry for 11 years. Darlene's focus has been in Implementation, Provider Network, and Relationship Management.

Office: 803-396-4784 | Email: Darlene.Grindstaff@keybenefit.com

GROUP IMPLEMENTATION OVERVIEW

Phase 1	Phase 2	Phase 3	Phase 4
Implementation Begins	Group Details - Open Enrollment, Eligibility Setup, ID Card, Banking, and Vendors	Finalize Setups - Open Enrollment, Eligibility, ID Card, Banking and Vendors	Confirmations - Fulfillment, Post Implementation Planning, GO LIVE!
☐ Identify key group contacts and their Roles and Responsibilities.	☐ Confirm Open Enrollment dates.	☐ Introduce Billing and Customer Service Teams.	☐ Confirm eligibility loaded.
☐ Review contracts and electronic signature process.	☐ Discuss materials needed for Open Enrollment and expected delivery.	☐ Process Open Enrollment eligibility.	☐ Confirm fulfillment mail date.
☐ Confirm Signor information.	☐ Discuss COBRA Administration. (if applicable)	☐ Finalize COBRA Administration setup. (if applicable)	☐ Confirm COBRA letter and packet mail dates. (if applicable)
☐ Establish meeting schedule, frequency and set target dates.	☐ Discuss Customer Service setup.	☐ Finalize EZ Benefits setup.	☐ Confirm all group setups are complete.
☐ Begin KBA Client Profile.	☐ Review EDI Setup and initial eligibility file submission.	☐ Finalize ID Card setup.	☐ Confirm plan document has been signed.
☐ Request prior plan document.	☐ Schedule EDI meeting. (if applicable)	☐ Finalize banking setups.	☐ Deliver 1st Months Invoice and finalize.
☐ Review plan document, benefits and discuss changes.	☐ Review EZ Benefits platform.	☐ Finalize claims and EOB setups.	☐ Deliver final reinsurance contract.
☐ Request claim file and accumulator data.	☐ Discuss ID Card setup, approval process and delivery expectations.	☐ Finalize PBM Implementation.	☐ Discuss reporting needs for plan year.
☐ Review Fee Schedule, Implementation Fee.	☐ Discuss Banking and Funding setup for monthly premiums and claims.	☐ Finalize Plan Document.	☐ Complete full transition to Client Relationship Team for administration.
☐ Discuss Notice of Termination to prior carrier and obtain copy.	☐ Discuss frequency of check runs, large dollar claims review, exception process and EOBs.	☐ Review status of EDI setup. (if applicable)	
	☐ Discuss monthly premium billing preferences.		
	☐ Begin PBM Implementation and schedule meetings.		
	☐ Discuss vendor setup process.		
	☐ Review special programs. (if		

Group Implementation and Client Relations Team

Group implementations are assigned members from the Implementation and Client Relations Team. KBA is committed to delivering a “WOW” level of customer service and our team will ensure a smooth and timely transition to KBA.

It is our priority to execute a successful implementation.

Group Implementation Leadership

- Implementation Director
 - Christine Vinesett
 - Email: Christine.Vinesett@keybenefit.com | Phone: (803) 323-7974
- Implementation Manager
 - Rachel Morris
 - Email: Rachel.morris@keybenefit.com | Phone: (803) 396-4775
- Implementation Lead
 - Ashley Gowen
 - Email: Ashley.gowen@keybenefit.com | Phone: (317) 284-7863
- ❖ Implementation Coordinator's will be assigned at kick-off.

Client Relations Leadership

- Senior Vice President, Client Relations
 - Nicole Oliver
 - Email: Nicole.oliver@keybenefit.com | Phone: (803) 396-4720
- ❖ Client Relations VP and Account Representative will be assigned at kick-off.

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	☐ Review special programs. (if applicable)		
Live Group Administration			

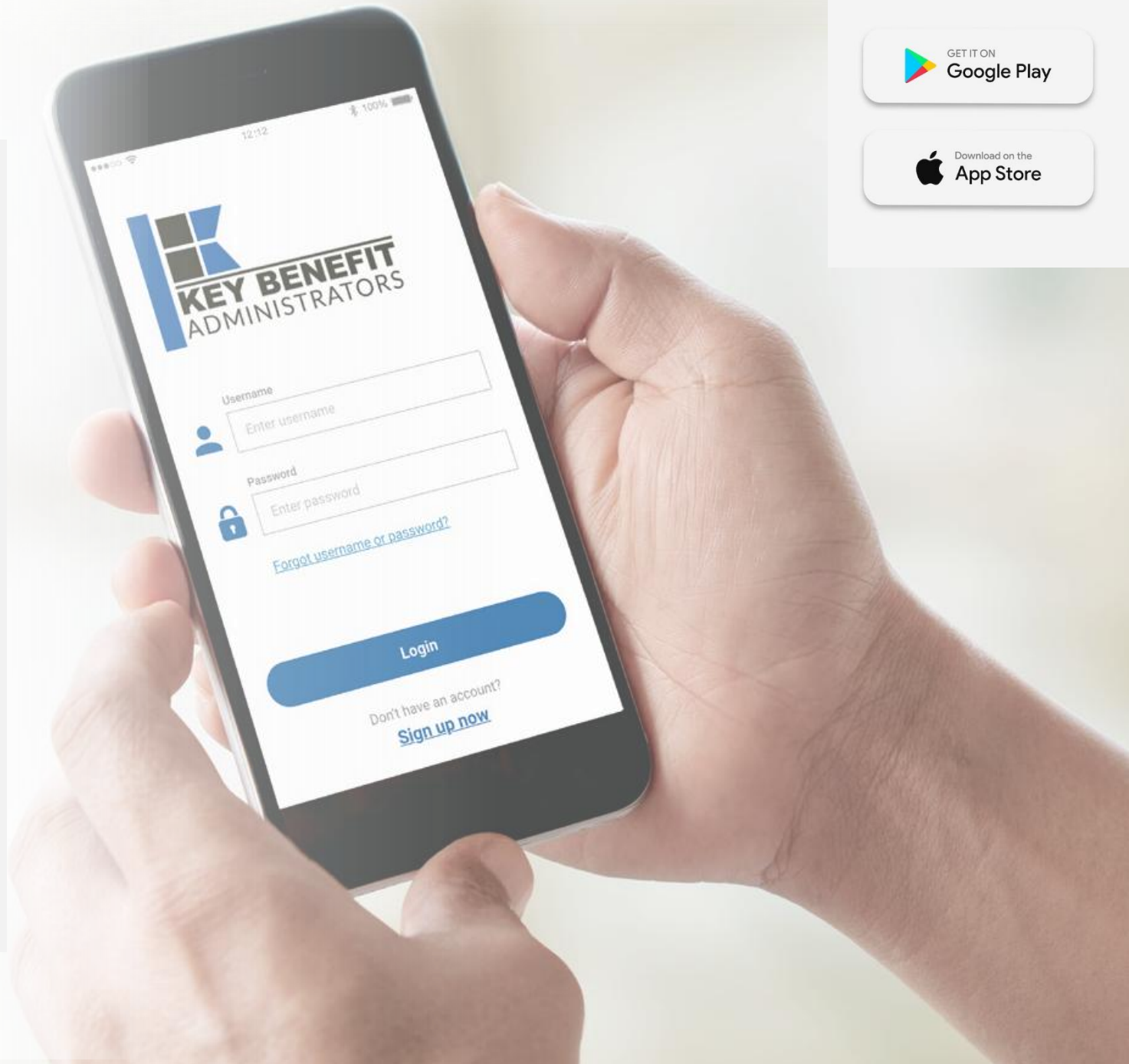
DTE IMPLEMENTATION OVERVIEW

Phase 1	Phase 2		Phase 3	
DTE Network Setup	DTE Marketing & Website Setup	DTE Provider File Setup	Group Sales & Plan Design Setup	Group Administration Setup
☐ Legal Contracts and Agreements	☐ DTE Name Established	☐ Provider data and file analysis	☐ Group Sales and Marketing Strategy	☐ Proposal system setup
☐ Security and Compliance review	☐ DTE logo and messaging	☐ Provider file requirements	☐ Group Sales collateral	☐ Group Implementation and Renewal Collateral
☐ Secure email connection - Transport Layer Security (TLS)	☐ DTE Marketing Collateral	☐ Provider file transfer	☐ Group plan design and benefit structure	☐ Administration system programming (DTE product, plan design, billing, distribution, extract files)
☐ Secure file and reporting connection - Secure File Transfer Protocol (SFTP)	☐ DTE Website URL (domain)	☐ Provider file programming and website testing	☐ PBM plans and benefits review	☐ Claims adjudication system programming
☐ Network review of providers, specialties and sub-specialties	☐ DTE Website design and content creation	☐ Provider maintenance file established	☐ PBM structure analysis (systems, data import, data export, billing and invoicing)	☐ Fulfillment programming
☐ Network contracting and pricing analysis	☐ DTE Website provider portal setup	☐ DTE Website review for launch	☐ PBM file structure programming and testing (claims, accumulators, elig, invoice feed)	☐ Customer Service programming
☐ Network fee schedule programming and testing			☐ Reinsurance Carrier education on DTE	☐ Complete End to End Testing
Network launch	Website launch		Group Sales launch	

Web Portal and Mobile Application

Gives you accessibility to your benefit plan information, claims data, eligibility details and much more!

- Biometric Login for ease of access (Fingerprint & Facial Recognition)
- Simplified Accumulator View
- Recent Claims Summary and Full Detail
- Benefit Information
- View/Request an ID Card (Display - Print – Fax – Email)
- SSO directly to Vendor Solutions
- Rx Price Comparison and Pharmacy Discount Programs (if applicable)
- Express Requests to Customer Support
- Push Notifications (Mobile App)



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Measuring Success: Customer Experience

Key Performance Indicators

Metric	2025 YTD	Industry Benchmark	How We Compare	
Abandonment Rate	1.04%	3–5% (acceptable), <2% (best-in-class)	Excellent 🏆	✓ Industry-leading responsiveness 1.04% abandonment and 17 s average speed of answer—minimal dropped calls, maximum access. ✓ Outstanding quality & access 90% of calls answered within 30s (vs. 80% target) and 99%+ audit scores across voice, email & chat. ✓ Balanced efficiency & depth Handle time reflects the detailed, compassionate TPA support your members need.
Average Speed of Answer (ASA)	17 seconds	<28 seconds (typical target); ~3 min in healthcare BPO	Excellent 🏆	
SLA (80% ≤ 30 sec)	90.00%	80% in 20–30 seconds (standard)	Above target ✅	
Average Handle Time	7 minutes 47 seconds	4–6 minutes typical; 7–8+ mins for complex TPA calls	Higher than industry, but expected	
After Call Work (ACW)	49 seconds	30–60 seconds typical	Within range 🟢	
Quality Audits (Email, Chat, Soft Skills, Procedural)	99%+	95–97% considered strong	Outstanding 100	

KBA DEDICATED TEAM ROLES

Our expert team is committed to delivering tailored solutions and exceptional service, ensuring consistent support and strategic guidance for every client.

Client Vice President (CVP)

Average Tenure: 15 Years

A dedicated individual delivering solutions tailored to client needs, driven by their unique data. The KBA CVP is deeply knowledgeable about client demographics, goals, and plans, as well as the health status of their membership. Through unparalleled analytics, the CVP illuminates the intelligence of the data, coordinates and conducts plan performance reviews, and provides meaningful recommendations. The CVP can also assist with employee enrollment meetings, presentations, and announcement materials, staying informed on client issues and initiatives, and tracking all account-related information.

Benefit Consultant (BC)

Average Tenure: 5 Years

Delivers strategic consultation to the client in conjunction with the CVP, with a primary focus on reporting and strategic insights. During renewal season, the BC assists in preparing comprehensive renewal proposals that include detailed service offerings, clear fee structures, and defined timelines. They also coordinate and conduct employee enrollment meetings as needed, ensuring a seamless and informed process for all stakeholders.

Account Representative (AR)

Average Tenure: 3 Years

Manages the day-to-day needs of the account, ensuring consistent communications, timely follow-up services, and continuous account monitoring. The AR oversees new case implementation and renewal processes alongside the KBA operations team. Responsibilities include the distribution of monthly reports, facilitating plan design changes, and obtaining signatures for plan documents and amendments. Additionally, they assist with funding management, including ACH transfer of funds and various other inquiries to ensure smooth financial operations.



ADDITIONAL KBA TEAM MEMBERS

On-Site KBA Support

An on-site resource for members to ask questions regarding benefits, eligibility, or claims. This role also provides essential HR support for support for plan administration, ensuring smooth operations and direct assistance.

Economist/Analyst

George Avery, PhD, MPA: Dr. Avery holds a doctorate in Health Services Research, Policy, and Administration from the University of Minnesota. With extensive academic experience as a professor at Purdue University and the University of Minnesota Duluth, he has contributed significantly to his field. His impressive publication record includes six book chapters and over 20 peer-reviewed journal articles.

Dr. Avery's expertise is highly sought after, evidenced by his advisory roles for prestigious organizations such as the Indiana Office of Rural Health, the Centers for Disease Control and Prevention, the Department of Defense, and the United States Environmental Protection Agency. He brings over 30 years of invaluable experience in public health, health economics, and econometric analysis to the team.



PLAN GROWTH MODELING



MODELING TRINITY HEALTH (MI) GO-TO-MARKET OPPORTUNITY

1

MODELING DEMAND

With over 10 years of experience, KBA has learned to accurately forecast plan growth, as a function of brand quality.

HCAHPS recommendation rates and competitive landscape are highly predictive of possible demand.

Trinity Health hospitals in MI have high variation in patient recommendation rates. Accordingly, some regions are far more likely to see growth.

2

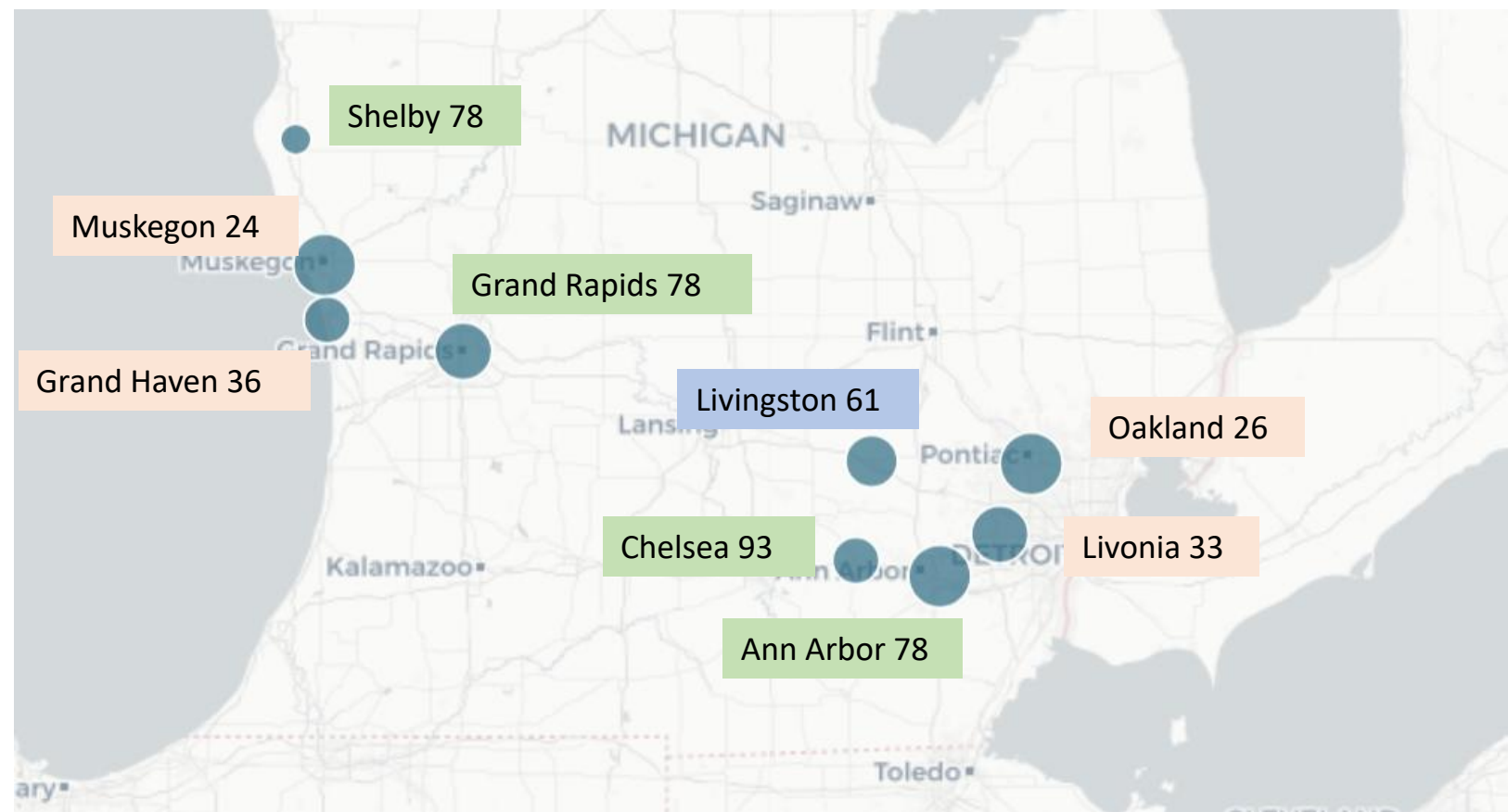
MODELING PROFITABILITY

KBA models increased revenue coming from increased patient steerage.

To convert revenue growth to profit, KBA estimates high, medium and low estimates for net income based on (1) estimates of commercial margin and (2) the within-DTE-network steerage rate.

VARIATION IN PATIENT HCAHPS RECOMMENDATIONS

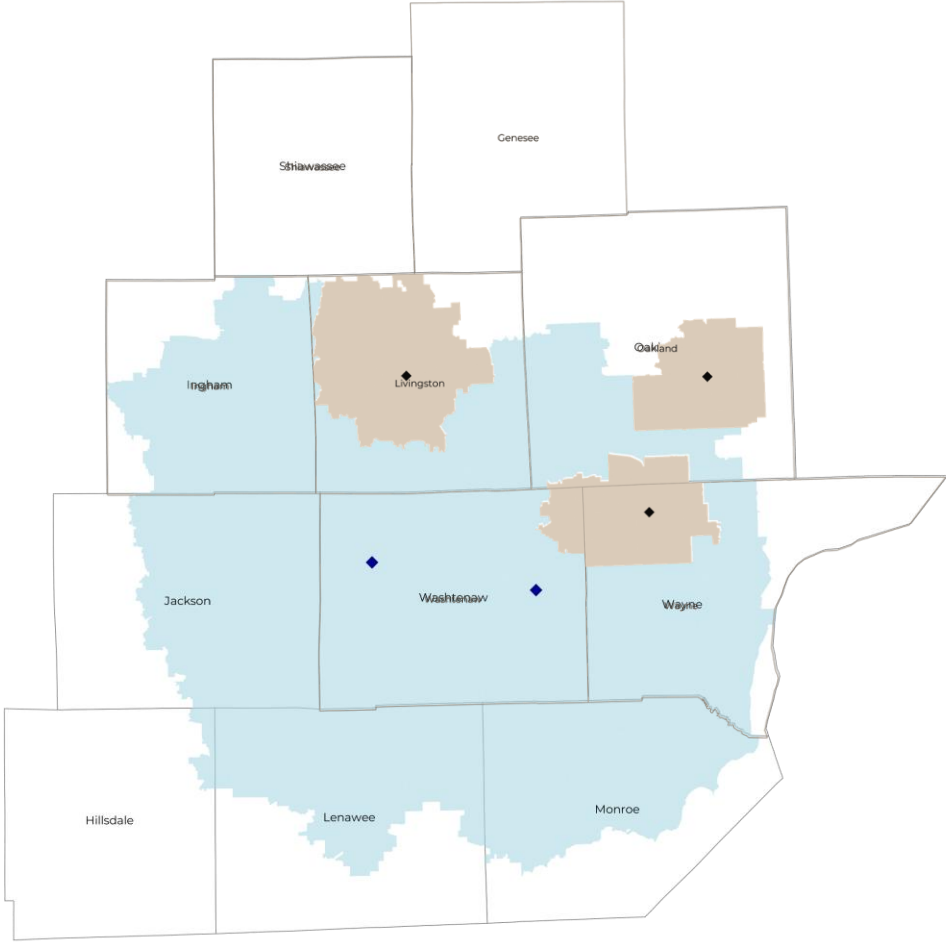
National HCAHPS Patient Recommendation Percentile by Hospital



KEY FINDING

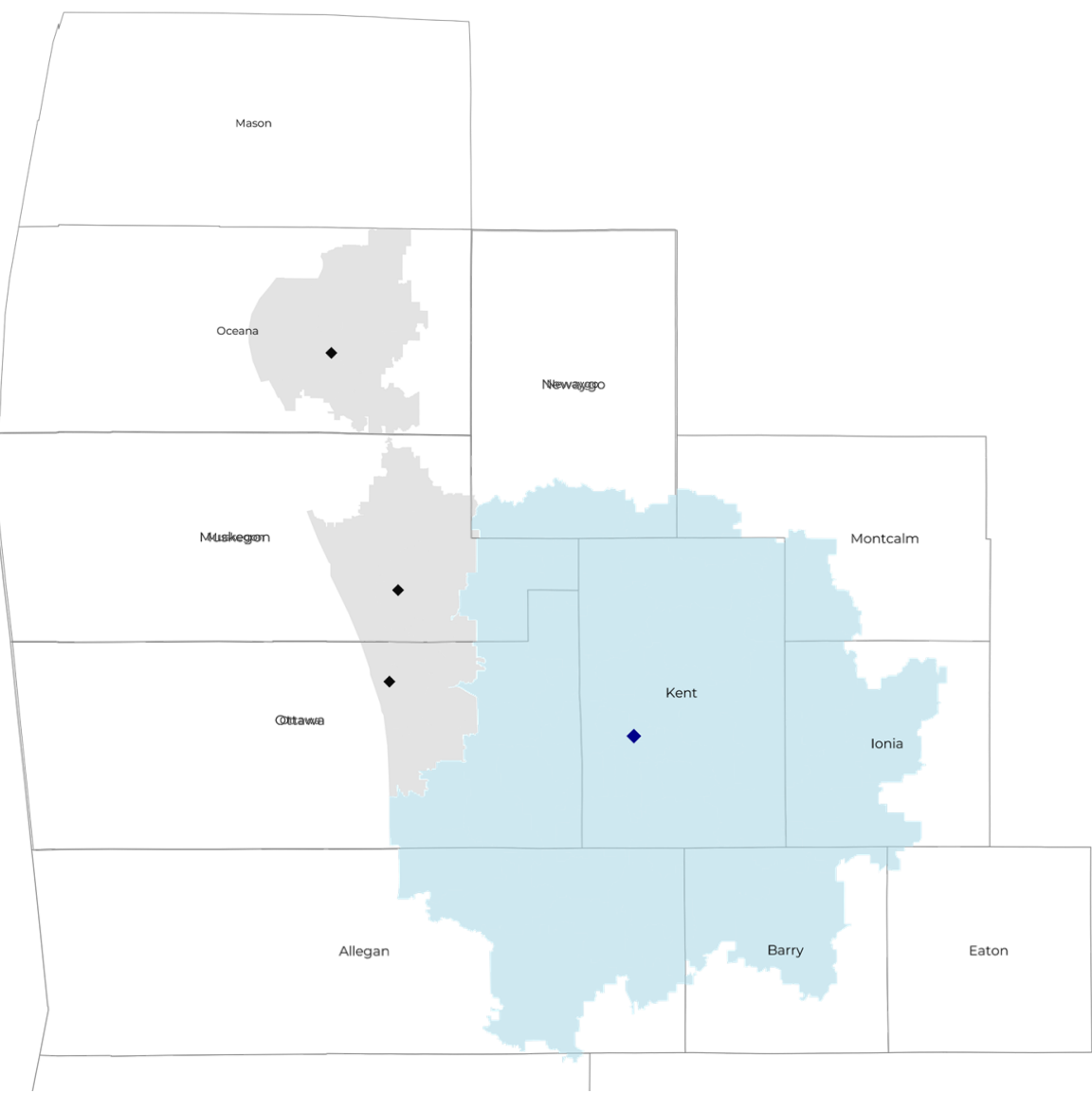
KBA models that Ann Arbor and Grand Rapids would fuel growth for a Michigan-wide Trinity Health DTE plan.

DETROIT/ANN ARBOR COMPETITIVE LANDSCAPE



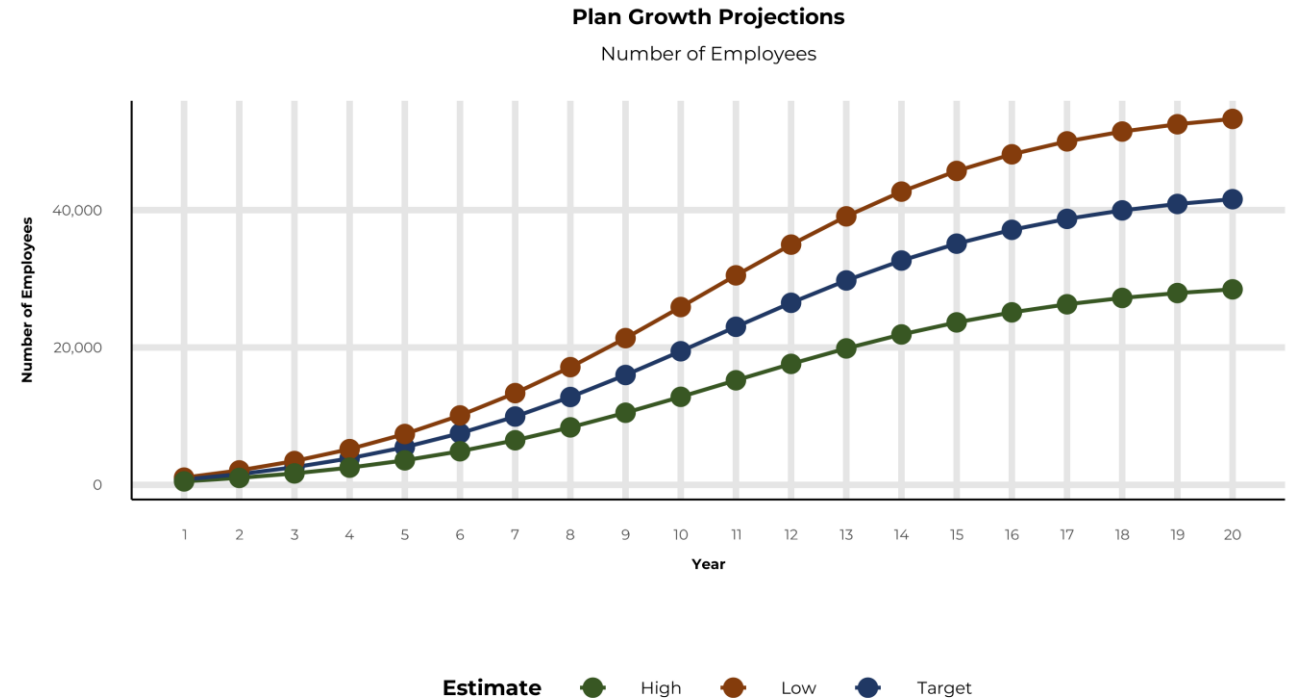
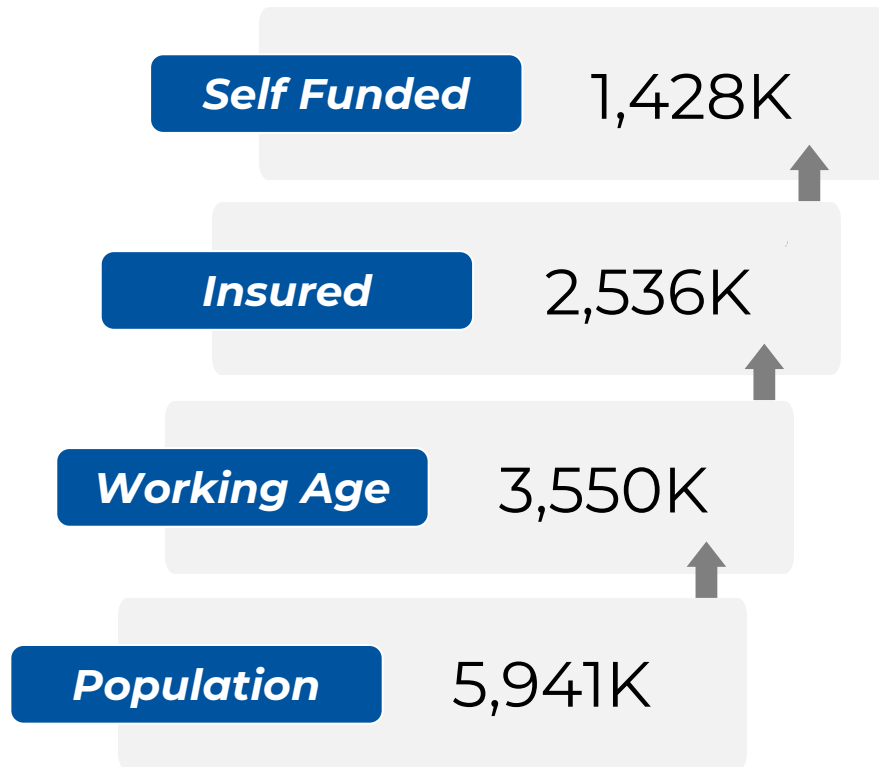
Health System	HCAHPS Recommendation Percentile	KBA Brand Quality Score
University of Michigan Health	89	74
Trinity Health (Ann Arbor and Chelsea)	85	73
McLaren Health Care Corporation	68	70
Department of Veterans Affairs	60	66
Trinity Health (Combined)	54	63
Ascension Health	43	60
Henry Ford Health	41	59
Trinity Health (Livingston, Oakland, Livonia)	40	58
Corewell Health	30	55
ProMedica Health System	15	50
Tenet Healthcare	3	46
Prime Healthcare Services	5	45

MUSKEGON/GRAND RAPIDS COMPETITIVE LANDSCAPE



Health System	HCAHPS Recommendation Percentile	KBA Brand Quality Score
Trinity Health Grand Rapids	78	73
University of Michigan Health	74	71
Other Independent Hospitals	72	70
Corewell Health	69	67
Trinity Health (All)	58	64
Trinity Health Muskegon, Grand Haven and Shelby	46	61

PLAN POTENTIAL GROWTH PATHS



Estimates includes the Trinity Health MI service area in Allegan, Barry, Genesee, Hillsdale, Ingham, Ionia, Jackson, Kent, Lapeer, Lenawee, Livingston, Macomb, Monroe, Montcalm, Muskegon, Newaygo, Oakland, Oceana, Ottawa, Shiawassee, Washtenaw, and Wayne Counties

ESTIMATED PLAN GROWTH BY REGION

Forecasted Number of Employees in Direct-to-Employer Plans

Year	Grand Rapids	Muskegon, Grand Haven and Shelby	Oakland, Livonia, Livingston	Ann Arbor, Chelsea	Total Employees	Total Members	Percent of Members Choosing Trinity Health MI for Inpatient Care	Percent of Members Choosing Trinity Health MI for Outpatient Care
1	400	0	100	300	800	1,440	40%	40%
2	800	0	100	600	1,500	2,700	43%	43%
3	1,200	100	200	1,000	2,500	4,500	45%	45%
4	1,900	100	300	1,600	3,900	7,020	46%	46%
5	2,700	100	500	2,200	5,500	9,900	47%	47%
6	3,600	200	700	3,000	7,500	13,500	48%	48%
7	4,800	200	900	4,000	9,900	17,820	49%	49%
8	6,200	300	1,100	5,200	12,800	23,040	50%	50%
9	7,800	400	1,400	6,500	16,100	28,980	50%	50%
10	9,500	400	1,700	7,800	19,400	34,920	51%	51%
11	11,200	500	2,000	9,300	23,000	41,400	52%	52%
12	12,900	600	2,300	10,700	26,500	47,700	53%	53%
13	14,500	700	2,600	12,000	29,800	53,640	54%	54%
14	15,900	700	2,800	13,200	32,600	58,680	55%	55%
15	17,100	800	3,000	14,200	35,100	63,180	55%	55%
16	18,000	800	3,200	15,000	37,000	66,600	56%	56%
17	18,800	800	3,400	15,600	38,600	69,480	57%	57%
18	19,400	900	3,500	16,100	39,900	71,820	58%	58%
19	19,900	900	3,600	16,500	40,900	73,620	58%	58%
20	20,200	900	3,600	16,800	41,500	74,700	59%	59%

STEERAGE GROW RATE ASSUMPTIONS

KBA estimates that each year a new cohort of employers joins the health plan. Of these employers, 40% have 15% of their employees already utilizing Trinity Health, 40% have 30% utilization, and 20% have 50% utilization.

After joining, employers experience an initial 12 percentage-point increase in Trinity Health utilization, followed by an additional 7 percentage-point increase each year until reaching a maximum of 85% utilization.

High estimates begin approximately 10 percentage points above the target, while low estimates are set 10 percentage points below the target.

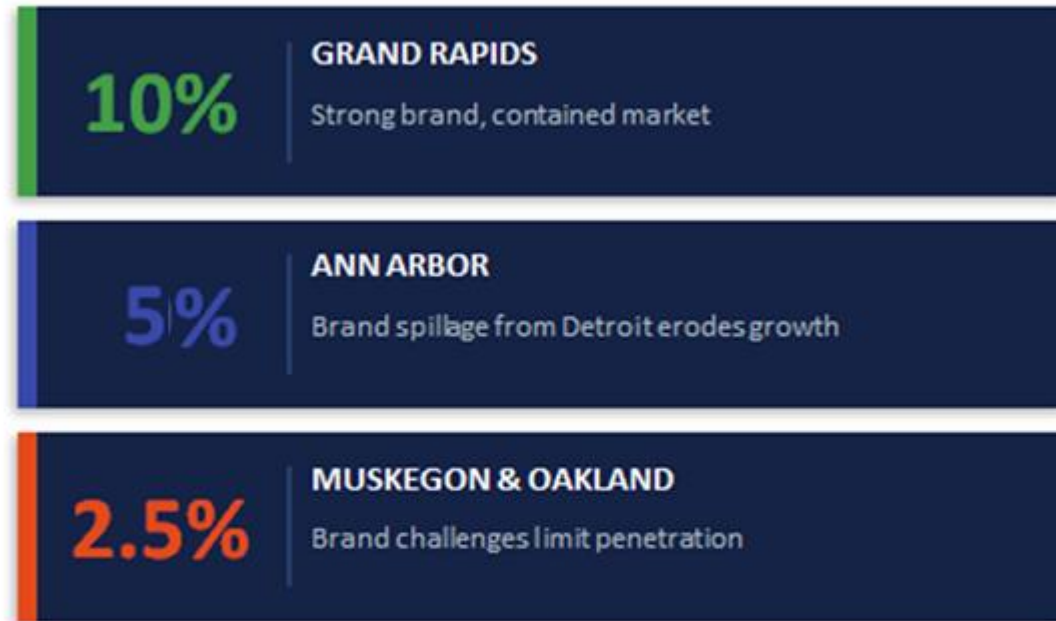
Parameter	Assumption / Value
STARTING TIER DISTRIBUTION	
Tier 1 — 15% Starting Steerage	40% of groups
Tier 2 — 30% Starting Steerage	40% of groups
Tier 3 — 50% Starting Steerage	20% of groups
STEERAGE GROWTH RULES	
Year 1 in Plan	+12 percentage-point bump
Each Subsequent Year	+7 percentage points / year
Maximum Cap	85% (steerage ceiling)
COUNTERFACTUAL ASSUMPTION	
Without-Plan Steerage	No increase — stays at starting tier level
Weighted Avg Starting Level	28%

ESTIMATING PLAN GROWTH

Equations and Estimates

MARKET PENETRATION AT MATURITY

% of market the plan could reach at full maturity:



RETENTION RATE ESTIMATES

Current KBA retention: 83% | Projection basis: 80%



S-CURVE GROWTH MODEL

$$\text{Employees}(t) = \text{Max} / (1 + e^{(-k \times (t - \text{midpoint}))})$$

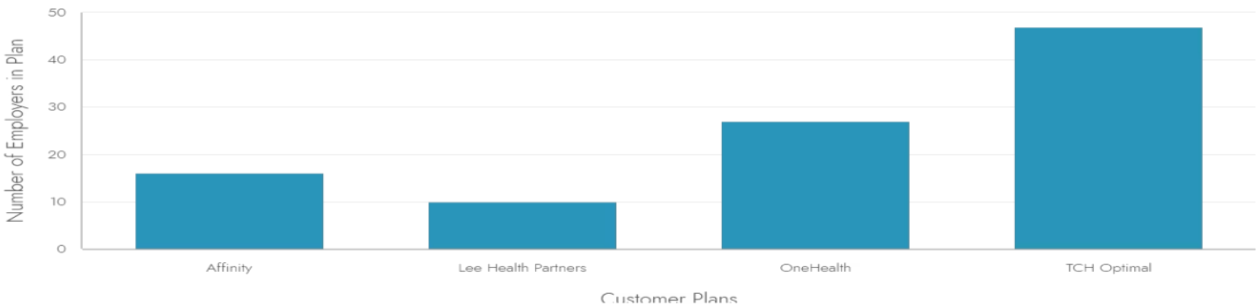
Validation: Growth curves mapped against current plan growth in established KBA markets to confirm accuracy.

80%+

STEERAGE RATES | Conservatively estimated using current established KBA plans as guides. Current steerage exceeds 80% within existing KBA networks.

KBA Plan Growth in the First Two Years

An average of 25 employer groups in plan within the first two years!



1,600

Louisville KY

OneHealth Plan achieved strong initial growth

1,800

Bentonville AR

Northwest Health Plan exceeded projections

6,400

Cincinnati OH

TCH Optimal demonstrated exceptional market acceptance

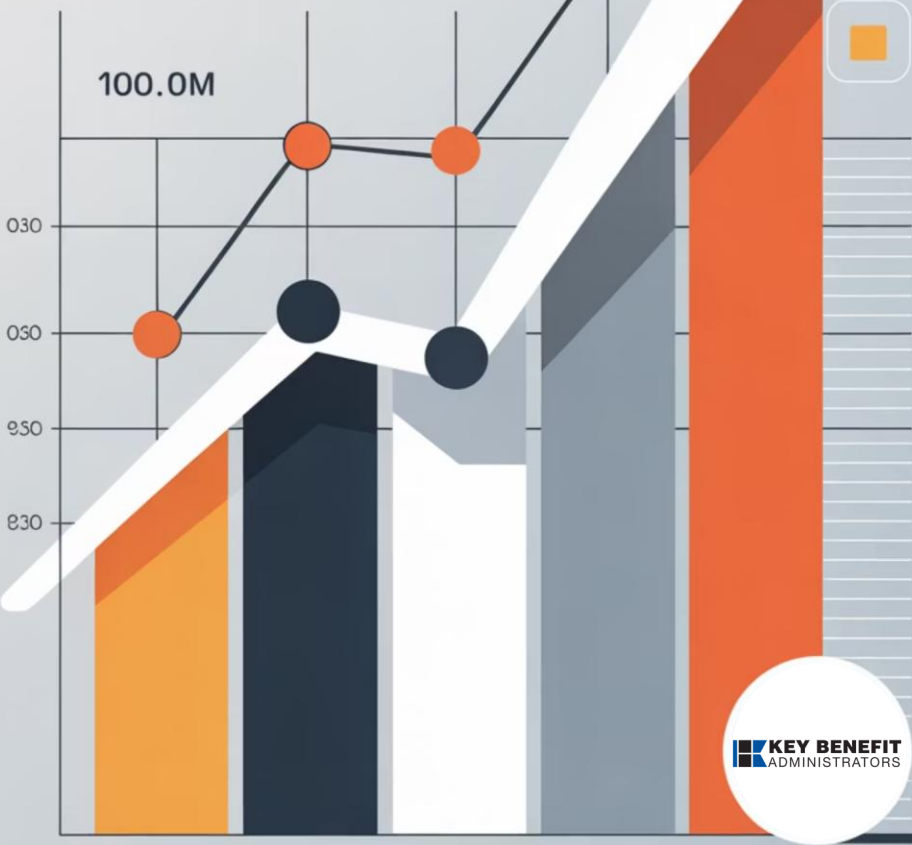
12,900

Fort Myers FL

Lee Health Care Partners achieved remarkable growth

These results demonstrate KBA's proven ability to build sustainable membership growth across diverse market conditions and geographic conditions and geographic regions. Each partnership reflects our commitment to long-term success rather than short-term enrollment term enrollment spikes.

Membership Growth



GROWING POWERFUL, LASTING PROFITABILITY

Early Stage

Gaining Name Recognition, Early Growth

New Revenue: **\$8.38M** New Profit: **\$0.95M**

Growth Stage

Strong Reputation, Strong Growth

New Revenue: **\$58.41M** New Profit: **\$7.77M**

Mature Stage

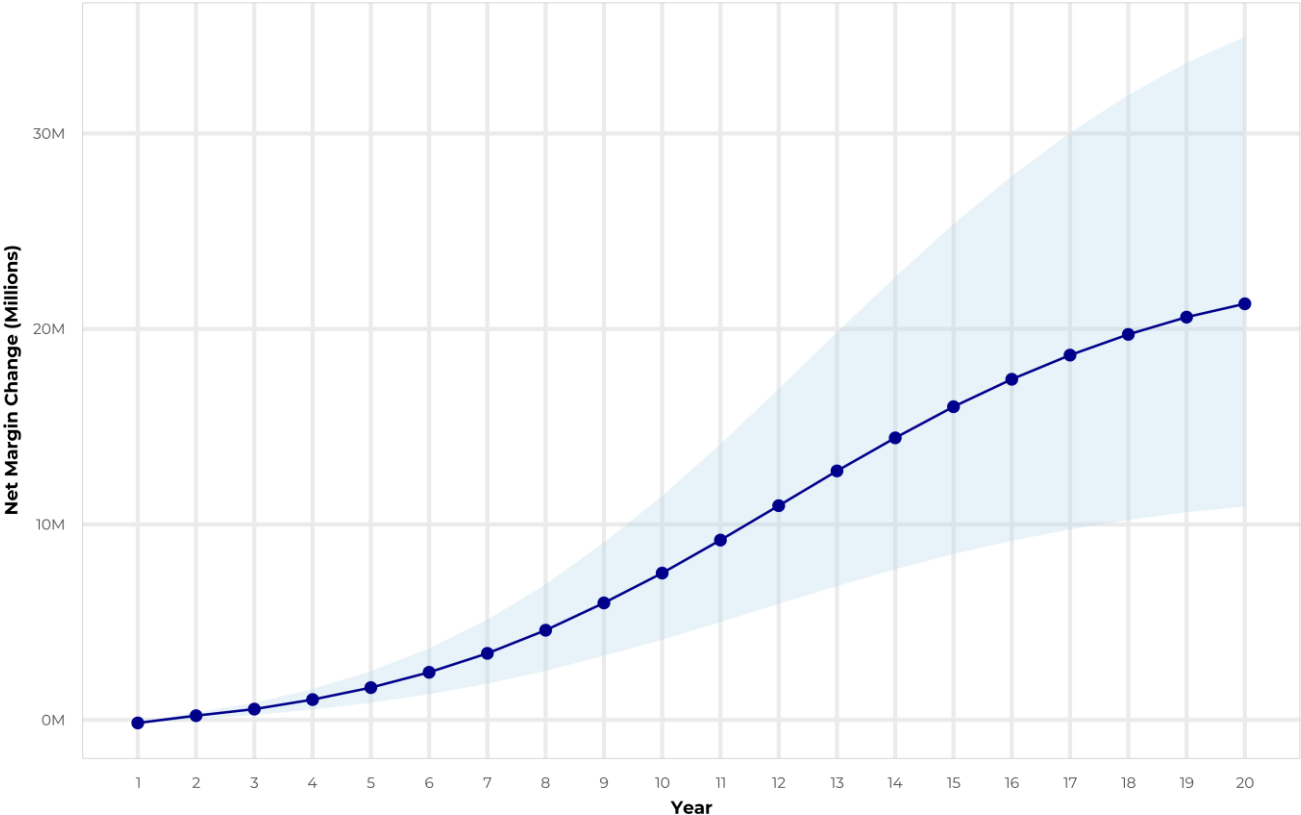
High Profitability, Market Saturation

New Revenue: **\$137.31M** New Profit: **\$18.31M**

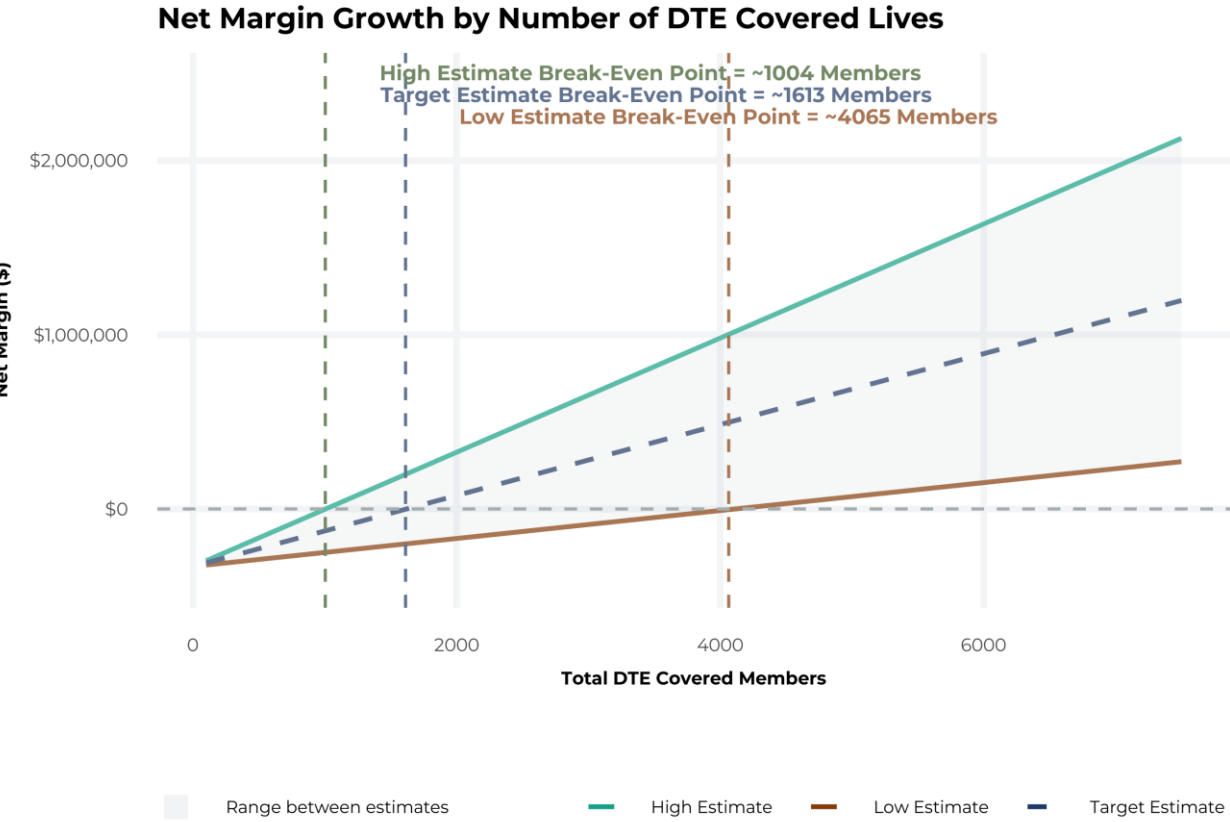
Net Margin Change Over Years

Variation by Member Growth With Target Estimate Assuming

50%+ Within-Plan Steerage and a Commercial Margin of 12.8%



MODEL ASSUMPTIONS AND BREAK EVEN



Patient Steerage Assumptions

KBA estimates that steerage for employers groups jumps by 12 percentage points in the first year and seven percentage points each year after. For further assumptions, review slide 50

Low	High
10% below target	10% above target

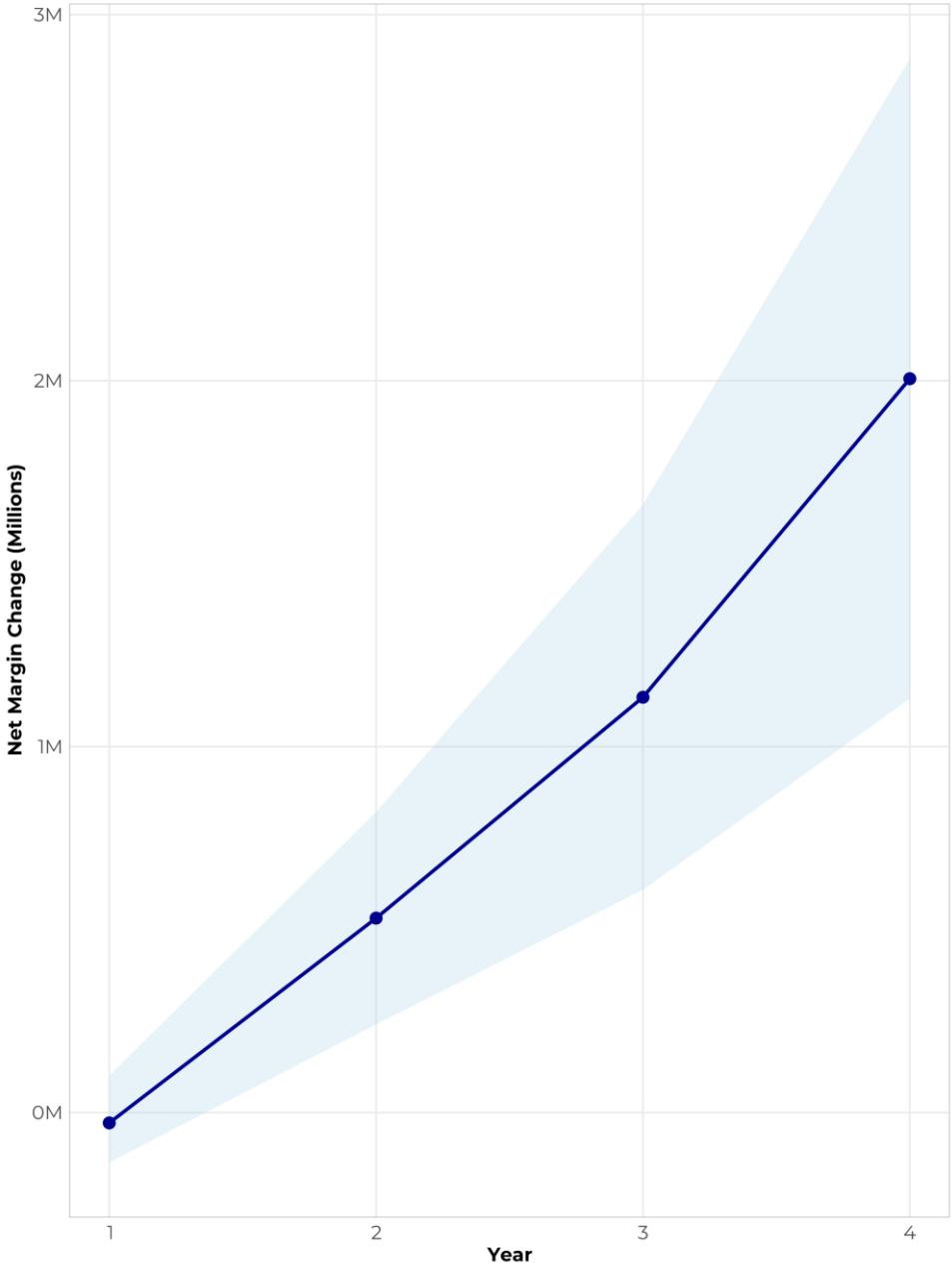
Commercial Margin Assumptions

KBA observes a Trinity Health MI 16% commercial margin. Conservatively, we model the DTE to be even with commercial rates given prices 10% below market, but with a 10 percentage-point increase through friendlier billing practices and improved patient collections with the patient card.

Low	Target	High
10.3%	12.8%	15.3%

FOUR-YEAR PROJECTIONS

Net Margin Change Over Years
Variation by Member Growth With Target Estimate Assuming
50%+ Within-Plan Steerage and a Commercial Margin of 12.8%



Target Member Growth				
Metric	Year 1	Year 2	Year 3	Year 4
Number of Members	1,400	2,700	4,500	7,000
Incremental Margin	285.0K	679.0K	1,261.0K	2,101.3K
Network Access Fee	16.8K	32.4K	54.0K	84.0K
Implementation Fee (One-Time)	-150.0K	0.0K	0.0K	0.0K
Sales and Marketing Costs	-180.0K	-180.0K	-180.0K	-180.0K
Net Margin Change	-28.2K	531.4K	1,135.0K	2,005.3K

High Member Growth				
Metric	Year 1	Year 2	Year 3	Year 4
Number of Members	2,000	3,800	6,300	9,800
Incremental Margin	407.2K	955.6K	1,765.4K	2,941.9K
Network Access Fee	24.0K	45.6K	75.6K	117.6K
Implementation Fee (One-Time)	-150.0K	0.0K	0.0K	0.0K
Sales and Marketing Costs	-180.0K	-180.0K	-180.0K	-180.0K
Net Margin Change	101.2K	821.2K	1,661.0K	2,879.5K

Low Member Growth				
Metric	Year 1	Year 2	Year 3	Year 4
Number of Members	900	1,600	2,700	4,200
Incremental Margin	183.2K	402.4K	756.6K	1,260.8K
Network Access Fee	10.8K	19.2K	32.4K	50.4K
Implementation Fee (One-Time)	-150.0K	0.0K	0.0K	0.0K
Sales and Marketing Costs	-180.0K	-180.0K	-180.0K	-180.0K
Net Margin Change	-136.0K	241.6K	609.0K	1,131.2K

DIRECT-TO-EMPLOYER STRATEGY:

ONLY AVAILABLE TO *PREMIER* HEALTH SYSTEMS



STRONG Brand Quality

EXTENSIVE Patient Access

AFFORDABLE Value Pricing

EXCELLENT Care Quality

TRINITY HEALTH'S NETWORK

- The medical providers in your direct control or part of Trinity Health's integrated delivery system
- Potential risk sharing arrangements (e.g. employer stop-loss participation)

FRIENDS & FAMILY PROVIDERS – TIER 1

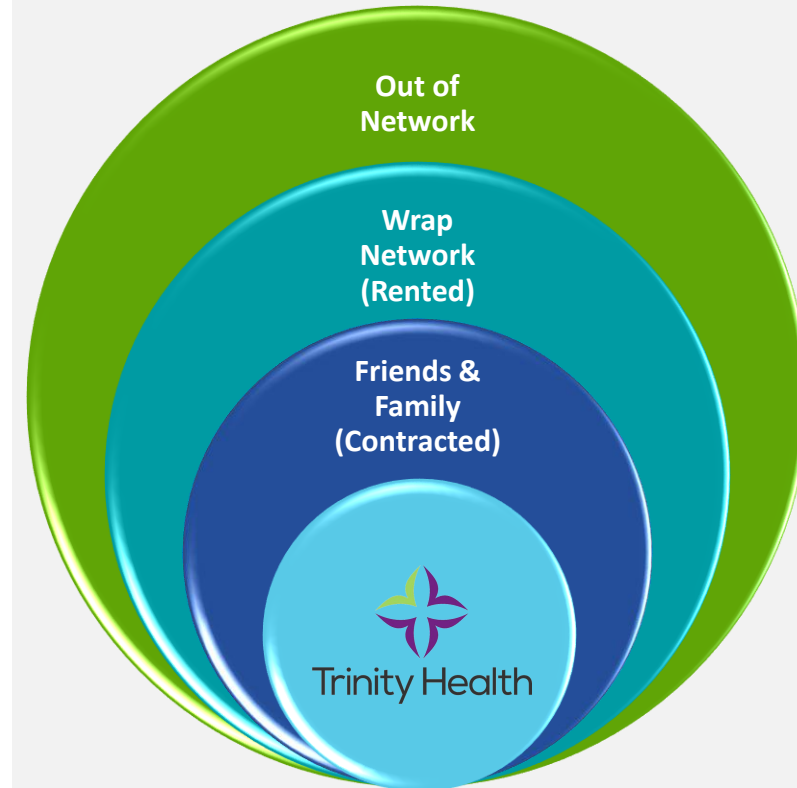
- Access through direct contracts at the discretion of Trinity Health

WRAP NETWORK

- Medical services from national PPO networks for members that travel, tertiary and specialty services you do not provide, and employees residing outside your direct catchment area

OUT-OF-NETWORK

- Non-contracted medical providers



DTE Covered Members Financial Analysis

Target Estimate Assuming 50%+ Within-Plan Steerage and a Commercial Margin of 12.8%.

DTE Members	Pre-DTE Revenue	Pre-DTE Margin	DTE Revenue	DTE Incr. Revenue	DTE Total Margin	DTE Incr. Margin	Net Margin Less DTE Fees
100	\$ 349K	\$ 45K	\$ 499K	\$ 150K	\$ 64K	\$ 19K	\$ -310K
500	\$1,746K	\$ 224K	\$ 2,495K	\$ 748K	\$ 319K	\$ 96K	\$ -228K
1,000	\$ 3,492K	\$ 447K	\$ 4,989K	\$1,497K	\$ 639K	\$ 192K	\$ -126K
2,500	\$ 8,731K	\$1,118K	\$12,473K	\$ 3,742K	\$1,597K	\$ 479K	\$ 179K
10,000	\$34,924K	\$4,470K	\$49,891K	\$14,967K	\$ 6,386K	\$1,916K	\$1,706K
20,000	\$69,848K	\$8,940K	\$99,782K	\$29,935K	\$12,772K	\$3,832K	\$3,742K

- Financial projections do not include 340B contribution margin, which could represent \$136 PMPM by optimizing 340B through Rx plan design, Trinity Health MI's approved site(s) with network integration, and development of Trinity Health MI's proprietary PBM program.



FINANCIAL PROJECTIONS & ASSUMPTIONS

ESTIMATION OF TRINITY HEALTH (MI) 2022 PAYER MIX

Payer Group	Total Inpatient Days	Revenue	Payer Mix
All Trinity Health MI	483,052	\$3,156,251K	100%
Medicare	125,102	\$601,391K	26%
Medicaid	21,715	\$342,048K	5%
Other Government Pay	0		
No-Pay, Charity Care	16,169	-\$99,724K	3%
Commercial/Self Pay	320,066	\$2,212,975K	66%

Values in blue are taken from the 2021 Medicare Provider Cost Report (<https://data.cms.gov/provider-compliance/cost-report/hospital-provider-cost-report/data>)

Values in yellow are taken from Definitive Healthcare data.

Total Medicare patient revenue is reported by CMS in the Medicare Provider Analysis and Review Report (MedPAR)

Estimating Payer Mix

- Medicare, Medicaid, and other government payer mixes are estimated as the percentage of inpatient days for each group divided by total inpatient days.
- No-pay and charity care is estimated as the percentage of no-pay/charity care compared to total overall revenue.
- Commercial/self pay is assumed to be the rest of the total payer mix share.

Estimating Total Inpatient Days

- Total inpatient days for 'No Pay' and 'Commercial' are estimated to be the payer mix estimates for "No Pay, Charity Care" and "Commercial/Self Pay" multiplied by the total number of inpatient days.

In other analysis, the outpatient payer mix is assumed to mirror the inpatient payer mix.

ESTIMATION OF MARGIN FOR COMMERCIAL VS. GOVERNMENT PAY

The RAND data for Trinity Health MI estimates commercial insurance reimbursements at 197% of Medicare Rates

Estimating Commercial Margin

(Calculated through counterfactual of assuming all CMS/Government pay was paying at commercial rates)

Commercial Revenue	\$2,212,975K
Gov-pay revenue at 197% current rate	\$1,854,947K
Total Revenue	\$4,067,922K
Total Costs	\$3,445,449K
Net Income	\$622,473K
	15%

Estimating Gov-Pay Margin

(Calculated through counterfactual of assuming all commercial pay was paying at Medicare rate)

Commercial Revenue	\$1,125,535K
Divided by 197%	
Gov-pay revenue	\$943,439K
Total Revenue	\$2,068,975K
Total Costs	\$3,445,449K
Net Income	\$ -1,376,475K
	-67%

ESTIMATING MARKET SHARE

Hospital Name	Number of Beds	Inpatient Total Charges	Outpatient Total Charges	Inpatient Percent of Market Total	Outpatient Percent of Market Total (Assuming 66% of Physicians Employed by Hospitals)
Trinity Health Livonia Hospital	239	\$ 679,229.3K	\$ 637,624.3K	2%	1%
Trinity Health Oakland Hospital	333	\$ 812,993.1K	\$ 750,349.0K	2%	1%
Trinity Health Grand Rapids Hospital	275	\$ 664,098.2K	\$ 984,692.1K	2%	1%
Trinity Health Muskegon Hospital	160	\$ 253,691.1K	\$ 323,834.6K	1%	<1%
Trinity Health Livingston Hospital	42	\$ 106,478.4K	\$ 630,174.5K	<1%	1%
Trinity Health Ann Arbor Hospital	475	\$1,533,330.2K	\$1,778,989.2K	4%	2%
Trinity Health Grand Haven	36	\$ 17,225.2K	\$ 134,060.2K	<1%	<1%
Chelsea Hospital	79	\$ 169,526.0K	\$ 532,024.5K	<1%	1%
Trinity Health Shelby Hospital	24	\$ 6,185.7K	\$ 78,715.8K	<1%	<1%

11%
Inpatient
Market
Share

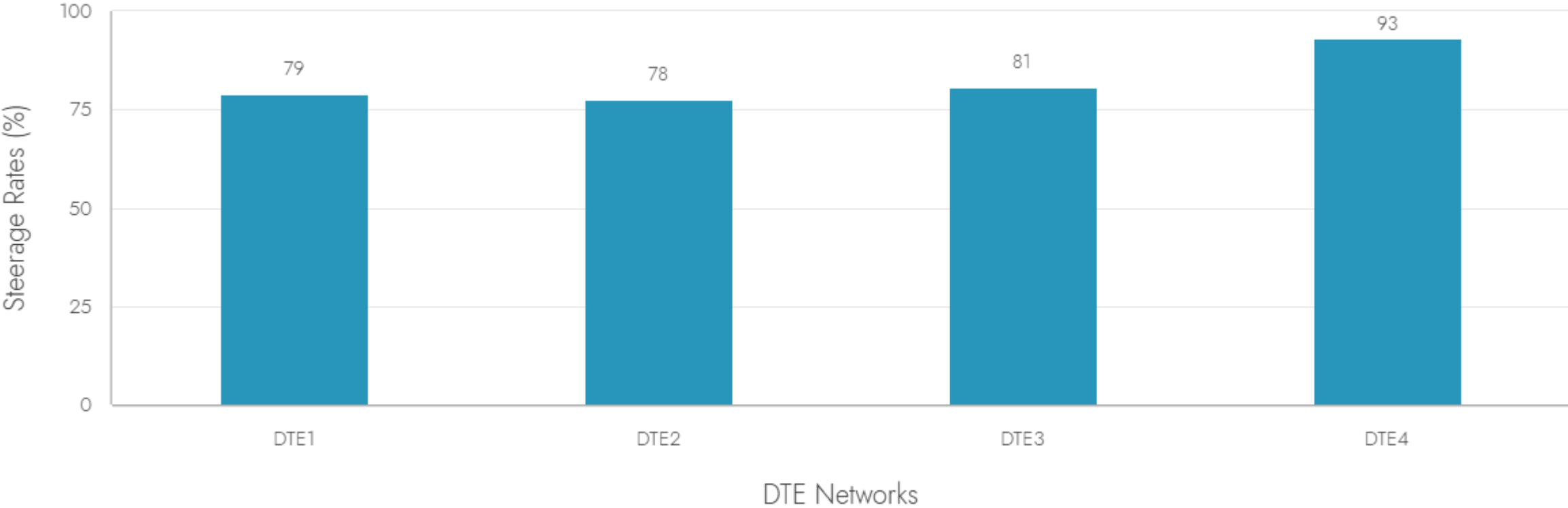
8%
Outpatient
Market
Share

For outpatient market share, it is estimated that only 66% of local physicians are employed by area hospitals (Research from the Physician Advocacy Institute:

<https://www.physiciansadvocacyinstitute.org/Portals/0/assets/docs/PAI-Research/PAI-Avalere%20Physician%20Employment%20Trends%20Study%202019-2023%20Final.pdf?ver=uGHF46u1GSeZgYXMKFyYvw%3d%3d>)

Hospital Metrics									
Metric	Trinity Health Livonia Hospital	Trinity Health Oakland Hospital	Trinity Health Grand Rapids Hospital	Trinity Health Livingston Hospital	Trinity Health Ann Arbor Hospital	Trinity Health Grand Haven	Chelse a Hospital	Trinity Health Shelby Hospital	Trinity Health Muskegon Hospital
Total Inpatient Patient Revenue	\$198,335K	\$249,589K	\$242,396K	\$28,962K	\$463,066K	\$4,944K	\$45,433K	\$3,050K	\$94,880K
Total Outpatient Patient Revenue	\$186,186K	\$230,357K	\$359,413K	\$171,407K	\$537,255K	\$38,475K	\$142,583K	\$38,807K	\$121,114K
Total Medicare Revenue	\$97,095K	\$105,320K	\$78,653K	\$35,920K	\$180,983K	\$3,819K	\$27,909K	\$2,603K	\$69,088K
Estimated Medicaid and Other Government-Pay Revenue	\$31,704K	\$61,565K	\$111,685K	\$11,623K	\$78,163K	\$5,709K	\$7,391K	\$5,684K	\$28,524K
Estimated Commercial Revenue	\$255,638K	\$313,618K	\$411,446K	\$152,889K	\$741,299K	\$33,837K	\$152,536K	\$33,593K	\$118,119K
Total Uncompensated Care	\$13,975K	\$17,510K	\$15,075K	\$6,331K	\$24,265K	\$1,639K	\$5,455K	\$2,313K	\$13,161K
Estimated Inpatient Costs	\$211,656K	\$267,985K	\$308,452K	\$24,554K	\$467,739K	\$5,878K	\$45,868K	\$2,676K	\$129,816K
Estimated Outpatient Costs	\$198,691K	\$247,336K	\$457,357K	\$145,318K	\$542,677K	\$45,746K	\$143,946K	\$34,047K	\$165,709K
Total Operating Expenses	\$410,347K	\$515,321K	\$765,808K	\$169,872K	\$1,010,416K	\$51,624K	\$189,814K	\$36,723K	\$295,524K
Net Income	\$-25,910K	\$-34,818K	\$-164,024K	\$30,560K	\$-9,970K	\$-8,258K	\$-1,978K	\$5,156K	\$-79,793K
Estimated Commercial Profit Margin	19.4%	19.7%	2.5%	31.0%	19.2%	1.8%	14.5%	26.4%	4.7%
1. Most data is taken from the CMS provider cost report									
2. Medicare costs are taken from the Medicare Provider Analysis and Review (MedPAR)									
3. Data with 'Estimated' in the title are derived from other data points.									
4. Market Share estimates based on total patient revenues with area hospitals. Ambulatory market share based on regional estimates of the percentage of local physicians employed by area hospitals. See Physician Advocacy Institute									

STEERAGE RATES FOR CURRENT DTE NETWORKS



The average Tier 1 Steerage is 83%.

YEAR 4 PROJECTED REVENUE AND PROFIT

Pre-DTE Cohort of 7,000 Employees	Inpatient	Outpatient	Professional	Total
Revenue†	\$ 6,666,626	\$ 12,120,072	\$ 5,659,970	\$ 24,446,668
Net Income‡	\$ 853,328.2	\$ 1,551,369	\$ 724,476.2	\$ 3,129,174

With DTE	Inpatient	Outpatient	Professional	Total
Revenue°	\$ 10,964,525	\$ 19,933,744	\$ 9,308,889	\$ 40,207,158
Net Income*	\$ 1,403,459	\$ 2,551,519	\$ 1,191,538	\$ 5,146,516

† Assuming proportional revenues that match initial group Trinity Health steerage (see slide 50).

‡ Conservatively assuming a commercial margin of 12.8% (80% of the estimated commercial margin)

° Assuming 46% inpatient and 46% market steerage (see slide 50 for assumptions)

* Conservatively assuming a commercial margin of 12.8%, with a 10 percentage-point reduction to reflect competitive-edge rates and a 10 percentage-point increase due to lower billing costs and higher patient responsibility collections because of the member convenience card.

Total Increase in Net Income	\$ 2,017,343
Network Access Fee (Revenue) of \$1 PMPM	\$ 84,000
Total Profit Growth	\$ 2,101,343
Minus Implementation Fee (One-Time)	-\$ 0
Minus Sales and Marketing (Annual – Ongoing)	-\$ 180,000
Total	\$ 1,921,343

Target Estimate Assuming 50%+ Within-Plan Steerage and a Commercial Margin of 12.8%				
All values in thousands of dollars. Modeled With Year 1 Fees and Patient Steerage.				
Metric	500 DTE Covered Lives	1000 DTE Covered Lives	5000 DTE Covered Lives	25000 DTE Covered Lives
Pre-DTE Group Inpatient Revenue	\$476.2	\$952.4	\$4,761.9	\$23,809.4
Pre-DTE Group Inpatient Margin	\$61.0	\$121.9	\$609.5	\$3,047.6
DTE Group Inpatient Revenue	\$680.3	\$1,360.5	\$6,802.7	\$34,013.4
DTE Group Inpatient Margin	\$87.1	\$174.1	\$870.7	\$4,353.7
DTE Group Incremental Inpatient Revenue	\$204.1	\$408.2	\$2,040.8	\$10,204.0
DTE Group Incremental Inpatient Margin	\$26.1	\$52.2	\$261.2	\$1,306.1
Pre-DTE Group Outpatient Revenue	\$865.7	\$1,731.4	\$8,657.2	\$43,286.0
Pre-DTE Group Outpatient Margin	\$110.8	\$221.6	\$1,108.1	\$5,540.6
DTE Group Outpatient Revenue	\$1,236.7	\$2,473.5	\$12,367.4	\$61,837.1
DTE Group Outpatient Margin	\$158.3	\$316.6	\$1,583.0	\$7,915.1
DTE Group Incremental Outpatient Revenue	\$371.0	\$742.0	\$3,710.2	\$18,551.1
DTE Group Incremental Outpatient Margin	\$47.5	\$95.0	\$474.9	\$2,374.5
Pre-DTE Group Professional Revenue	\$404.3	\$808.6	\$4,042.8	\$20,214.2
Pre-DTE Group Professional Margin	\$51.7	\$103.5	\$517.5	\$2,587.4
DTE Group Professional Revenue	\$577.5	\$1,155.1	\$5,775.5	\$28,877.4
DTE Group Professional Margin	\$73.9	\$147.9	\$739.3	\$3,696.3
DTE Group Incremental Professional Revenue	\$173.3	\$346.5	\$1,732.6	\$8,663.2
DTE Group Incremental Professional Margin	\$22.2	\$44.4	\$221.8	\$1,108.9
Net Margin Less DTE Fees	\$-228.2	\$-126.4	\$687.9	\$4,759.6

Target Estimate Assuming 50%+ Within-Plan Steerage and a Commercial Margin of 12.8%				
All values in thousands of dollars. Modeled With Year 1 Fees and Patient Steerage.				
Metric	500 DTE Covered Lives	1000 DTE Covered Lives	5000 DTE Covered Lives	25000 DTE Covered Lives
Pre-DTE Group Total Revenue	\$1,746.2	\$3,492.4	\$17,461.9	\$87,309.5
Pre-DTE Group Total Margin	\$223.5	\$447.0	\$2,235.1	\$11,175.6
DTE Group Total Revenue	\$2,494.6	\$4,989.1	\$24,945.6	\$124,727.9
DTE Group Incremental Total Revenue	\$748.4	\$1,496.7	\$7,483.7	\$37,418.4
Incremental Total Revenue by Member	\$1.5	\$1.5	\$1.5	\$1.5
DTE Group Total Margin	\$319.3	\$638.6	\$3,193.0	\$15,965.2
DTE Group Incremental Total Margin	\$95.8	\$191.6	\$957.9	\$4,789.6
Wellness Share	\$6.0	\$12.0	\$60.0	\$300.0
Implementation Fee (One-Time)	\$150.0	\$150.0	\$150.0	\$150.0
Sales and Marketing Annual Fee	\$180.0	\$180.0	\$180.0	\$180.0
Net Margin Less DTE Fees	\$-228.2	\$-126.4	\$687.9	\$4,759.6

High Estimate Assuming 60%+ Within-Plan Steerage and a Commercial Margin of 17.8%				
All values in thousands of dollars. Modeled With Year 1 Fees and Patient Steerage.				
Metric	500 DTE Covered Lives	1000 DTE Covered Lives	5000 DTE Covered Lives	25000 DTE Covered Lives
Pre-DTE Group Total Revenue	\$1,748.4	\$3,496.8	\$17,484.2	\$87,420.9
Pre-DTE Group Total Margin	\$223.8	\$447.6	\$2,238.0	\$11,189.9
DTE Group Total Revenue	\$2,494.6	\$4,989.1	\$24,945.6	\$124,727.9
DTE Group Incremental Total Revenue	\$746.1	\$1,492.3	\$7,461.4	\$37,307.0
DTE Group Total Margin	\$381.7	\$763.3	\$3,816.7	\$19,083.4
DTE Group Incremental Total Margin	\$157.9	\$315.7	\$1,578.7	\$7,893.5
Wellness Share	\$6.0	\$12.0	\$60.0	\$300.0
Implementation Fee (One-Time)	\$150.0	\$150.0	\$150.0	\$150.0
Sales and Marketing Annual Fee	\$180.0	\$180.0	\$180.0	\$180.0
Net Margin Less DTE Fees	\$-166.1	\$-2.3	\$1,308.7	\$7,863.5

Low Estimate Assuming 40%+ Within-Plan Steerage and a Commercial Margin of 7.8%				
All values in thousands of dollars. Modeled With Year 1 Fees and Patient Steerage.				
Metric	500 DTE Covered Lives	1000 DTE Covered Lives	5000 DTE Covered Lives	25000 DTE Covered Lives
Pre-DTE Group Total Revenue	\$1,741.0	\$3,482.0	\$17,409.9	\$87,049.7
Pre-DTE Group Total Margin	\$222.8	\$445.7	\$2,228.5	\$11,142.4
DTE Group Total Revenue	\$2,494.6	\$4,989.1	\$24,945.6	\$124,727.9
DTE Group Incremental Total Revenue	\$753.6	\$1,507.1	\$7,535.6	\$37,678.2
DTE Group Total Margin	\$256.9	\$513.9	\$2,569.4	\$12,847.0
DTE Group Incremental Total Margin	\$34.1	\$68.2	\$340.9	\$1,704.6
Wellness Share	\$6.0	\$12.0	\$60.0	\$300.0
Implementation Fee (One-Time)	\$150.0	\$150.0	\$150.0	\$150.0
Sales and Marketing Annual Fee	\$180.0	\$180.0	\$180.0	\$180.0
Net Margin Less DTE Fees	\$-289.9	\$-249.8	\$70.9	\$1,674.6

COLLABORATION OF STAKEHOLDERS



Key Benefit Administrators

- Full TPA Administration
- Stop-Loss Administration
- True Transparency & Analytics
- Population Health Management
- Utilization & Case Management
- Sales & Marketing
- Transparent PBM
- Website & Mobile App
- Customizable options & plans



Trinity Health's DTE

- Incremental Revenues and Margin
- Branding & Messaging
- Benefit Plan Integration
- Network Steerage
- Direct Relationship with Self-Funded Employers
- Referral Authorization
- Plan Transparency
- Direct Access to Members



Employer

- Reduced Spend
- "At The Table" with Trinity Health
- Improved Analytics
- Complete Transparency
- Lower Stop-Loss
- Benefit Plan Stabilization
- Healthier Population



Employee

- Reduced Out of Pocket
- Improved Health
- Better Coordination & Navigation of Care
- Direct Access to the Best Medical Providers

ROLES & RESPONSIBILITIES

SERVICES & ADMINISTRATIVE SYSTEMS PROVIDED BY:	
Trinity Health MI	KBA Operations
▪ Tier 1 Provider Network ▪ Provider Recruitment – If Necessary ▪ Network Pricing & Contract Terms ▪ Referral Management Protocols & Approvals ▪ Medical Director Oversight ▪ Practice Improvement Program using KBA/AHDI Data with Assistance from KBA	▪ Network Evaluation – Actuarial Pricing ▪ PPO Wrap & Tertiary Transplant Network, if needed ▪ Claims Administration ▪ Plan Design Development, Testing, and Administration ▪ Employer/Member/Provider Customer Service ▪ Customized Member/Provider/Agent/HR Portal ▪ Billing, Enrollment (Electronic/Paper), and Premium Management ▪ Fulfillment – SPD & Customized ID Cards ▪ COBRA/HIPAA Administration ▪ Out of Network Negotiations ▪ Wrap PPO Repricing ▪ Nurse Coaching for Chronically Ill ▪ Nurse Triage to Physicians and Facilities ▪ Stop-Loss Underwriting, Pricing, and Placement ▪ Stop Loss Claims Administration ▪ Complex Case Management ▪ PBM Administration Interface ▪ Reporting • Utilization & Gaps in Care • Clinically Based Physician Profiling • Referrals • Chronic Disease Management ▪ Employer Health Risk Management Reports ▪ Ancillary Administration – Dental, Vision, Weekly Income, 5500, Life, etc. ▪ Health Savings Account (HSA), Flexible Spending (FSA), Health Reimbursement Account (HRA) Administration ▪ Sales & Marketing Plan • Strategy Development: Branding, Brochures, Web Site • Distribution System Development • Sales Management
KBA Consulting	
▪ Program oversight and integration across the enterprise ▪ Goal development, agreement and tracking ▪ Start-up assessments: market, network, operational and readiness ▪ Draft business plan with financial projections ▪ Facilitate program communication and engagement with Trinity Health departments including system c-suite, operations, HR, managed care, marketing, strategy and business development	



PRE-IMPLEMENTATION

PROVIDER FILE

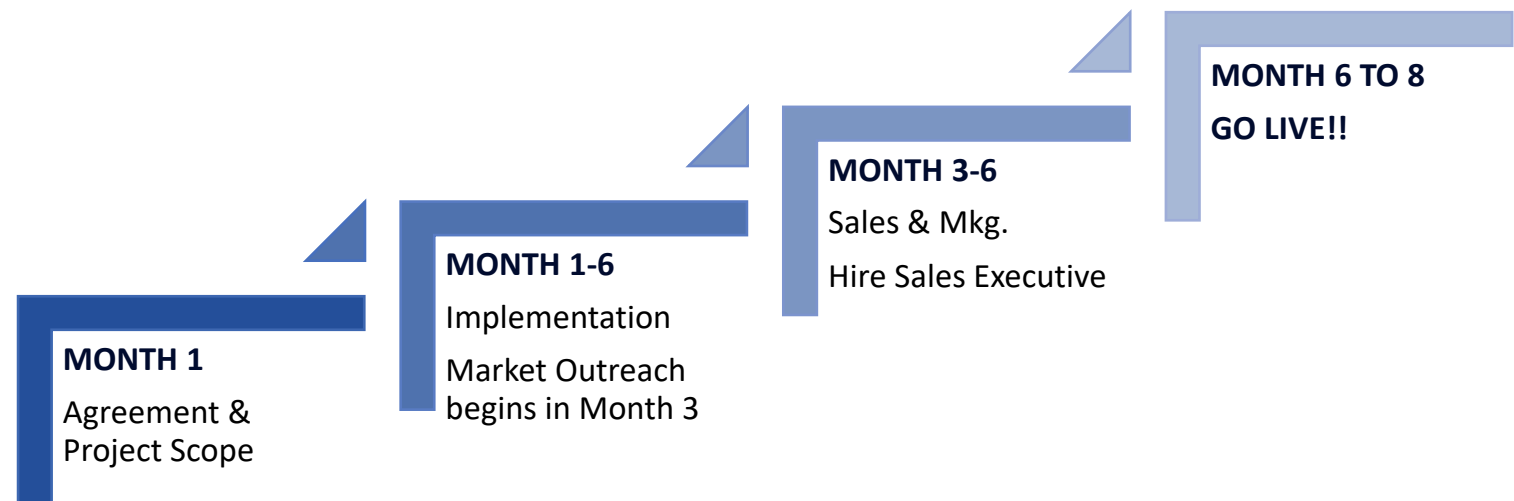
- Determine Gaps in Medical Service Delivery
- Identify Other Key Providers to Potentially Contract with on Behalf of Trinity Health
- Wrap Network to Cover All Other Provider Types

PRICING

- To Dial in Competitive Stop-Loss Rates and Trinity Health's Net Revenues/Margins
- Actuarial Evaluation Compared to the Market



IMPLEMENTATION



A PARTNER YOU CAN TRUST



**OUR OBJECTIVES
ARE ALIGNED**



**WE KNOW THE
MARKET**



**COMPLETELY
TRANSPARENT**



**WE HAVE THE
SOPHISTICATION
AND
INFRASTRUCTURE
TO EFFECTIVELY
COMPETE**



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